



## Life Care Plan Professionals, LLC

**David Glazer MD**

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Board Certified: Physical Medicine & Rehabilitation

Board Certified: Brain Injury Medicine

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### **Life Care Plan and Medical Cost Analysis**

#### **FINAL REPORT**

**Life Care Plan Completion Date: 09/15/2025**

**Name: xxxx**

**Age: 53**

**DOB: [REDACTED]**

**DOI: 06/11/2023**

**Future Medical Damages: \$1,343,540**

#### Abbreviations:

|     |                            |     |                            |
|-----|----------------------------|-----|----------------------------|
| ADL | activities of daily living | LOC | loss of consciousness      |
| CT  | computerized tomography    | MBB | medial branch block        |
| DOI | date of injury             | MRI | magnetic resonance imaging |
| DTR | deep tendon reflexes       | NCV | nerve conduction velocity  |
| ED  | emergency department       | PT  | physical therapy           |
| EMG | electromyography           | ROM | range of motion            |
| ESI | epidural steroid injection | S&F | slip and fall              |

### **Synopsis:**

Ms. Xxxx is a 53-year-old female who was injured in a slip and fall (S&F) on DOI 06/11/2023. The review of the medical documentation and doctor and patient interviews reveal that Ms. Xxxx sustained significant injuries to the right knee, lower back, cervical spine, head, and buttocks. Since the incident, Ms. Xxxx has had physical therapy, cemented right total knee arthroplasty using the Smith and Nephew Journey II system, right knee manipulation under anesthesia, cervical trigger point injections to the right trapezius, left levator scapulae, left and right semispinalis muscles, cervical trigger point injections to the right trapezius, left levator scapulae, left and right semispinalis muscles, left L5-S1 and S1-S2 transforaminal epidural steroid injection under fluoroscopic guidance, left L5-S1 and S1-S2 transforaminal epidural steroid injection under fluoroscopic guidance, and ongoing use of medication. Ms. Xxxx will continue to have additional medical care needs and the following is a summary of reasonably anticipated future care costs.

This Life Care Plan (LCP) is based on a detailed review of the medical records (outlined below). In addition, Dr. Glazer interviewed Ms. Xxxx on 08/18/2025 and did a clinical interview with Dr. Mark Emas, Neurology on 09/08/2025. All of Dr. Glazer's opinions are given within a reasonable degree of medical probability (RDMP), a qualitative measure based on education, training, skill, professional experience treating patients, and board certification in a specialty that is primarily related to the treatment of injuries. It is understood that an LCP is a dynamic document and that updates may be necessary in the future based on changes in the clinical pathway.

### **Credentials:**

This report was prepared by David Glazer, MD, a physician who is board certified in Physical Medicine and Rehabilitation with additional fellowship training and certification in Brain Injury Medicine. He has served as the medical director of the Central Virginia VA Health Care System's Polytrauma Rehabilitation Center caring for veterans and active duty servicemembers who have sustained various injuries including brain injuries. Dr. Glazer currently serves as a physician and Acting Chief of Physical Medicine and Rehabilitation at the Cincinnati VAMC caring for veterans with various injuries both neurologic and non-neurologic including traumatic and non-traumatic brain injury, stroke, spinal cord injury, long-covid, multiple sclerosis, amputations, chronic pain, and diseases of the joints and muscles in both the outpatient and inpatient consultative settings. He is licensed to practice medicine in multiple states, is board certified in (1) Physical Medicine and Rehabilitation and (2) Brain Injury Medicine. Dr. Glazer is engaged in active clinical practice of medicine in his specialty of (1) Physical Medicine and Rehabilitation and (2) Brain Injury Medicine, 90% of his professional time.

### **Medical Records Reviewed:**

Date Received – 08/19/25 Number of Pages – 1809 (Including Additional Records)

Dates of Visits – From 10/21/05 to 04/09/25

**Pre-Injury:**

10/21/2005 - Edward Young, M.D. - Procedure - (Pdf 17: p. 169)

04/27/2012 - Carole L. Neuman, M.D./Memorial Hospital Jacksonville - (Pdf 16: p. 15-16, 12-13)

04/27/2012 - Carole L. Neuman, M.D. -Procedure - Memorial Hospital Jacksonville - (Pdf 16: p. 76-77)

01/27/2013 - Todd Boring, M.D. /Memorial Hospital Jacksonville - (Pdf 16: p. 290-298)

01/27/2013 - 01/28/2013 - Nam Phuong T. Lu, D.O. /Memorial Hospital Jacksonville - (Pdf 16: p. 308-309, 310-311, 284-285)

01/27/2013 - Abdul R. Kani, M.D. /Memorial Hospital Jacksonville - (Pdf 16: p. 312-315)

01/27/2013 - Jay E. Xxxx, M.D. – Radiologist - Memorial Hospital Jacksonville - (Pdf 16: p. 358)

01/27/2013 - Steven Kailes, M.D./Memorial Hospital Jacksonville - (Pdf 16: p. 361, 363)

01/27/2013 - Lawrence Kanter, M.D./Memorial Hospital Jacksonville - (Pdf 16: p. 362, 364)

01/28/2013 - Salil Patel, M.D. /Memorial Hospital Jacksonville - (Pdf 16: p. 365-366, 359-360)

02/16/2014 - Michael L. Cordova, M.D./Memorial Hospital Jacksonville - (Pdf 16: p. 434-439)

02/16/2014 - Scott Kaltman, M.D. – Radiologist - Memorial Hospital Jacksonville - (Pdf 16: p. 449)

02/16/2014 - Andrew McBride, M.D. – Radiologist - Memorial Hospital Jacksonville - (Pdf 16: p. 450)

02/25/2014 - 05/02/2023 - Edward Young, M.D. /Jacksonville Orthopaedic Institute - (Pdf 17: p. 165-168, 160-162, 140-142, 134-136, 130-132, 127-129, 124-126, 121-123, 117-119, 112-114, 106-108, 101-103, 94-96)

02/26/2014 - Joshua Yellin, M.D. – Radiologist - Precision Imaging Centers - (Pdf 17: p. 158-159)

03/10/2014 - 03/24/2014 - Barbara Short /Jacksonville Orthopaedic Institute - (Pdf 17: p. 152-154, 147-148, 145-146, 143-144)

10/14/2016 - Zwinda Ortiz Roldan, M.D./First Coast Cardiovascular Institute - (Pdf 16: p. 472-474)

10/14/2016 - Midmark Diagnostic Group - (Pdf 16: p. 661)

11/07/2016 - Arundhati Rao, M.D. - (Pdf 16: p. 470-471)

11/09/2016 - Arundhati Rao, M.D. - Procedure - Memorial Hospital Jacksonville - (Pdf 16: p. 539-540)

11/10/2016 - Arundhati Rao, M.D./Memorial Hospital Jacksonville - (Pdf 16: p. 483-486)

11/11/2016 - 11/12/2016 - Steven Webb, M.D./Memorial Hospital Jacksonville - (Pdf 16: p. 481-482, 478-480, 467-469)

04/16/2019 - David Estle, M.D. -Radiologist - Precision Imaging Centers - (Pdf 17: p. 105)

08/26/2021 - Kamal Bohsali, M.D. /Jacksonville Orthopaedic Institute - (Pdf 17: p. 97-99)

06/30/2022 - 09/01/2022 - Yomari Rodriguez, D.C., Doctor of Chiropractic/Advanced Healthcare Center - (Addl 2: Pdf 1: p. 40-43, 39, 38, 36, 35, 34, 33, 32, 31, 30, 29, 28, 27, 26)

07/05/2022 - 07/14/2022 - Jesse Badger, MS, PA-C/Advanced Healthcare Center - (Addl 2: Pdf 1: p. 6-12, 1-4)

07/19/2022 - 10/04/2022 - F. Carter Wilson, D.C., Doctor of Chiropractic/Advanced Healthcare Center - (Addl 2: Pdf 1: p. 37, 25, 23-24, 22, 21, 20)

08/24/2022 - Soheil Sooudi, M.D. - Radiologist - Advanced Healthcare Center - (Addl 2: Pdf 1: p. 16-19)

01/26/2023 - 05/18/2023 - Robert Hurford, M.D./Southeast Orthopedic Specialists - (Addl 2: Pdf 4: p. 20-23, 18-19, 8-11)

03/14/2023 - Katherine McLeod, PA-C/Southeast Orthopedic Specialists - (Addl 2: Pdf 4: p. 15-17)

04/13/2023 - Isaac Gooding, M.D. - Procedure - Southeast Orthopedic Specialists - (Addl 2: Pdf 4: p. 12-14)

### **Post-Injury:**

06/13/2023 - Jill A. Ward, M.D. /HCA Florida Memorial Hospital - (Pdf 11: p. 13-23)

06/13/2023 - James McClellan, M.D. – Radiologist - HCA Florida Memorial Hospital - (Pdf 11: p. 32-33)

06/13/2023 - Jeffrey West, M.D. – Radiologist - HCA Florida Memorial Hospital - (Pdf 11: p. 34)

06/13/2023 - Sean Kuyper, M.D. – Radiologist - HCA Florida Memorial Hospital - (Pdf 11: p. 35)

06/21/2023 - Stacey Shelton, APRN /Millennium Physician Group - (Pdf 1: p. 1)

06/29/2023 - Robert Hurford, M.D./Southeast Orthopedic Specialists - (Addl 2: Pdf 4: p. 4-7)

07/13/2023 - 02/26/2024 - Edward Young, M.D. /Jacksonville Orthopaedic Institute - (Pdf 17: p. 89-91, 66-68, 58-60, 41-43, 31-33, 12-14)

07/19/2023 - Bryan Howze, M.D. – Radiologist - Jacksonville Orthopaedic Institute - (Pdf 17: p. 82)

07/19/2023 - 08/16/2023 - Edward Young, M.D. – Procedure - Jacksonville Orthopaedic Institute - (Pdf 17: p. 83-87, 61-62)

07/28/2023 - 03/15/2024 - David Doward, M.D./Jacksonville Orthopaedic Institute - (Pdf 17: p. 77-80, 34-37, 23-25, 16-18, 7-10)

08/01/2023 - 09/05/2023 - Anika Goel, M.D./EMAS Spine and Brain Specialists - (Pdf 10: p. 1-3, Pdf 9: p.4-7)

08/02/2023 - Richard Newman, M.D. /EMAS Spine and Brain Specialists - (Pdf 9: p. 10-11)

08/11/2023 - Steven Pruett, DPT/Brooks Rehabilitation - (Pdf 17: p. 70-75)

08/23/2023 - Thomas Chu, M.D. – Radiologist - Insite Radiology - (Pdf 9: p. 8-9)

09/07/2023 - 02/01/2025 - Shahid M. Nasir, M.D. – Radiologist - Monument-9A Imaging Center - (Pdf 2: p. 44-45, 37-39)

10/10/2023 - 10/21/2024 - EMAS Spine and Brain Specialists - (Pdf 6: p.195-197, 157-160, 106-109, 69-72)

10/10/2023 - 11/12/2024 - Mark Emas, M.D. /EMAS Spine and Brain Specialists - (Pdf 6: p.200-204, 198-199, 181-185, 176-179, 169-173, 163-168, 161-162, 140-145, 134-139, 127-133, 120-126, 112-119, 110-111, 88-95, 21-29, 11-18, 51-58, 75-76, 1-10, 97, 98-101)

11/29/2023 - 10/21/2024 - Mark Emas, M.D. – Procedure - EMAS Spine and Brain Specialists - (Pdf 6: p.188, 77)

12/18/2023 - John Hegarty III, M.D. – Radiologist - Monument-9A Imaging Center - (Pdf 2: p. 40-41)

02/19/2024 - Olga Guskova, PA-C/EMAS Spine and Brain Specialists - (Pdf 6: p.151-156)

04/01/2024 - Gregory Keller, M.D./Jacksonville Orthopaedic Institute - (Pdf 17: p. 4-6)

07/30/2024 - Stephen Kramarich, M.D. /EMAS Spine and Brain Specialists - (Pdf 6: p.102-105)

04/09/2025 - Dashmire Ramadani, M.S., BCBA/River Shores Counseling - (Addl 1: Pdf 1: p. 1-3)

### Treating Doctor's Diagnoses:

| Date       | Provider/Location                                        | Diagnoses                                                                                                                                                                                                             |
|------------|----------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 06/13/2023 | Jill A. Ward, M.D./HCA Florida Memorial Hospital         | Closed head injury, back strain, closed head injury with brief loss of consciousness, and knee contusion. (Pdf 11: p. 13-23)                                                                                          |
| 06/21/2023 | Stacey Shelton, APRN /Millennium Physician Group         | Postconcussional syndrome, concessional with loss of consciousness, and lumbar sprain. (Pdf 1: p. 1)                                                                                                                  |
| 07/13/2023 | Edward Young, M.D. /Jacksonville Orthopaedic Institute   | Severe DJD of the right knee, history of right shoulder impingement with type III acromion, status post left rotator cuff repair, status post right hip arthrodesis, obesity, and right knee pain. (Pdf 17: p. 89-91) |
| 07/28/2023 | David Doward, M.D. /Jacksonville Orthopaedic Institute   | Lower back pain, left sciatica, and sacroiliitis. (Pdf 17: p. 77-80)                                                                                                                                                  |
| 08/01/2023 | Anika Goel, M.D. /EMAS Spine and Brain Specialists       | Mild cognitive impairment, dizziness, and headaches. (Pdf 10: p. 1-3)                                                                                                                                                 |
| 08/15/2023 | Edward Young, M.D. /Jacksonville Orthopaedic Institute   | Arthrofibrosis post-cemented right total knee arthroplasty (Smith & Nephew implants). (Pdf 17: p. 66-68)                                                                                                              |
| 11/07/2023 | David Doward, M.D. /Jacksonville Orthopaedic Institute   | Lumbar stenosis with neurogenic claudication and lumbar radiculopathy. (Pdf 17: p. 34-37)                                                                                                                             |
| 02/19/2024 | Olga Guskova, PA-C/EMAS Spine and Brain Specialists      | Cervical disc herniation, cervical spondylosis, cervical radiculitis, lumbar disc herniation, cervicogenic headache. (Pdf 6: p. 151-156)                                                                              |
| 04/01/2024 | Gregory Keller, M.D. /Jacksonville Orthopaedic Institute | Left-sided sciatic pain due to collapse of L5-S1, causing NF narrowing. (Pdf 17: p. 4-6)                                                                                                                              |

### General Information:

Current living situation - Ms. Xxxx is a 53-year-old single woman who denies smoking and drinking.

Language(s) – Ms. Xxxx speaks English. (Pdf 11: p. 2)

### **Incident:**

On 06/11/2023, Ms. Xxxx was showering at an Embassy Suites Hotel. According to her account, the shower began to back up, creating a hazardous condition. She subsequently slipped and fell backward in the tub, striking the back of her head. She believes she may have lost consciousness, as her first recollection following the fall was her mother entering the room and informing her that she had just fallen. (Pdf 6: p. 200)

Per Ms Xxxx, “the only thing I remember is hearing my Mom knocking on the door and calling my name.” It seems there is a likely lost of consciousness of 5 min or less. Her mom was staying in the same room as her.

### **Life Care Plans versus Treating Doctor Medical Records:**

A treating doctor’s note is usually a snapshot of a patient's current medical status and treatment, on the other hand, a life care plan is a detailed roadmap for addressing a patient's comprehensive care needs over their lifetime. There are several limitations to the use of treating doctor’s notes when formulating a life care plan. A treating doctor's medical note documents a specific focused patient encounter, examination, or in general the short-term treatment plan. Treating doctor’s notes are usually based on immediate medical judgment. The treating doctor’s notes tend to be a snapshot or memorandum of what the doctor is thinking about the treatment of the patient at that moment in time. The note is usually reviewed the next time the doctor sees the patient to quickly orient the treating doctor to his previous assessment of the patient.

However, according to the American Academy of Physician Life Care Planners (AAPLCP) the LCP is a document to discuss what “*medically-related goods and services does the condition require to:*

1. *Diminish or eliminate physical and psychological pain and suffering.*
2. *Reach and maintain the highest level of function given an individual’s unique circumstance.*
3. *Prevent complications to which an individual’s unique physical and mental conditions predispose them.*
4. *Afford the individual the best possible quality of life in light of their condition.”*

*Physician’s Guide to Life Care Planning, AAPLCP, p. 17, (2017).*

### **Treatment and Surgeries Related to Condition:**

06/13/2023 - Jill A. Ward, M.D./HCA Florida Memorial Hospital - Emergency Department Report

Subjective: Ms. Xxxx presents to the ED; she slipped and fell in the shower due to poor drainage, landing on her back and hitting her head. Her mother witnessed the incident and reported a brief loss of consciousness. The patient also reports worsened right knee pain, for which she is scheduled to have a knee replacement. She remains ambulatory. CT of the brain was performed by James McClellan, M.D., and an x-ray of the lumbar spine was performed by Sean Kuyper, M.D. were reviewed by Jill A. Ward, M.D.

Exam: Mild right anterior knee pain.

Assessment: Closed head injury, back strain, closed head injury with brief loss of consciousness, and knee contusion.

Life Care Plan - [REDACTED]

Plan: The patient is prescribed methocarbamol and meloxicam. The patient is advised to follow up with Zwinda S. Ortiz Roldan, M.D.

Condition: Stable.

Disposition: Discharge. (Pdf 11: p. 13-23)

06/21/2023 - Stacey Shelton, APRN /Millennium Physician Group

Subjective: The patient presented for follow-up. On 06/13/2023, she fell in the shower, twisted her right knee, and her back. The patient has complaints of right knee pain, back pain, and headache.

Exam: None on files.

Assessment: Postconcussional syndrome, concessional with loss of consciousness, and lumbar sprain.

Plan: The patient is referred to a neurologist and an orthopedic surgeon. (Pdf 1: p. 1)

06/29/2023 - Robert Hurford, M.D./Southeast Orthopedic Specialists

Subjective: The patient presented with complaints of lumbar spine pain. The pain was rated as 3/10 today in the clinic and 4-5/10 at its most severe. The pain was described as radiating down the left lower extremity and aggravated by activities like walking long distances.

Exam: The physical examination revealed moderate limitations in lumbar spine motion and pain with movement.

Assessment: Lower back pain, spondylosis without myelopathy, lumbosacral radiculopathy, and lumbar spondylosis.

Plan: A Left L5 TFESI was discussed as a future option if symptoms persist or worsen. The patient was scheduled to follow up in 2 months to assess progress. (Addl 2: Pdf 4: p. 4-7)

07/13/2023 - 02/26/2024 - Edward Young, M.D./Jacksonville Orthopaedic Institute

Subjective: The patient presented for a follow-up visit with a complaint of right knee pain. The pain is rated as an 8/10. The pain is described as sharp, throbbing, stabbing, burning, intermittent, and achy. The pain is aggravated by activity. The patient reports feeling worse.

Exam: The physical examination of the right knee revealed varus deformity with weight-bearing; tenderness at the medial joint line and crepitus at the patellofemoral joint; limited range of motion with loss of full extension and flexion.

Diagnostic Studies: An x-ray of the right knee was performed and reviewed.

Assessment: Severe DJD of right knee (post failed conservative treatment and cortisone injection on 09/14/2020), history of right shoulder impingement with type III acromion (failed conservative treatment), status post left rotator cuff repair (2 years ago), status post right hip arthrodesis, obesity, and right knee pain.

Plan: The patient is scheduled to undergo total knee arthroplasty. An x-ray of the right knee is ordered on 07/13/2023. The patient followed up for five sessions. The last session was on 02/26/2024. (Pdf 17: p. 89-91)

07/28/2023 - David Doward, M.D. /Jacksonville Orthopaedic Institute - Evaluation Note

**Subjective:** The patient presents with left-sided lower back and buttock pain radiating down the left leg to the foot, with numbness and weakness in an S1 distribution. The pain has been ongoing since a fall in the shower on 06/11/2023. She has a history of lower back issues previously treated with epidural steroid injections by Dr. Hurford in July 2022. The pain is rated as an 8/10, significantly impacting her daily activities.

**Exam:** There is tenderness to palpation noted at the left side sacroiliac joint. The results are positive for the Slump test and the straight leg raise test.

**Assessment:** Lower back pain, left sciatica, and sacroiliitis.

**Plan:** An MRI of the lumbar spine is ordered on 07/28/2023. The patient is prescribed Medrol Dosepak. The patient is recommended to perform physical therapy. The patient followed up for a total of four sessions. The last session was on 03/15/2024. (Pdf 17: p. 77-80)

08/01/2023 - 09/05/2023 - Anika Goel, M.D. /EMAS Spine and Brain Specialists.

**Subjective:** The patient presented for a follow-up with the complaints of loss of consciousness, persistent memory issues, stating that she often cannot recall recent events and needs to concentrate harder to perform tasks at work.

**Exam:** The physical examination reveals mild to moderate cervical spine tenderness and spasm with decreased range of motion in flexion and extension. The thoracic spine showed mild tenderness and spasm. The lumbosacral spine exhibited mild to moderate tenderness and spasm with a limited range of motion on flexion and extension. Both left and right sacroiliac joints were tender with mild to moderate spasm.

**Assessment:** Concussion, mild cognitive impairment, dizziness, headaches, and postconcussion syndrome.

**Plan:** The patient is prescribed nortriptyline, rizatriptan, and meclizine. MRI of the brain is ordered. The patient followed up on 09/05/2023. (Pdf 10: p. 1-3)

08/11/2023 - Steven Pruett, DPT /Brooks Rehabilitation - Physical Therapy Initial Evaluation Note

**Subjective:** The patient is presented for an initial evaluation with a complaint of right knee pain. The pain is described as persistent, sharp, and achy and rated as 5/10 at rest and 10/10 at worst. The pain is aggravated by walking, standing, sit-to-stand transitions, stair climbing, and squatting.

**Objective:** The physical examination reveals palpation reveals tenderness in the right quadriceps, semitendinosus, semimembranosus, biceps femoris, and medial/lateral tibiofemoral joint line.

**Assessment:** None on file.

**Plan:** The treatment plan included therapeutic exercises, neuromuscular re-education, manual therapy, hot/cold therapy, and therapeutic activities two times a week for 12 weeks. (Pdf 17: p. 70-75)

08/15/2023 - Edward Young, M.D. /Jacksonville Orthopaedic Institute

**Subjective:** The patient presented for a follow-up for a right total knee arthroplasty. The pain is rated as 4/10 and 7/10 at worst. The pain is described as throbbing and achy, intermittent, and activity-related. The patient is emotionally distressed and states she has been crying frequently.

**Exam:** The physical examination of the right knee revealed mild-to-moderate diffuse swelling.

**Diagnostic Studies:** An x-ray of the right knee was performed and reviewed.



Assessment: Arthrofibrosis post-cemented right total knee arthroplasty (Smith & Nephew implants), history of right shoulder impingement with type III acromion and failed conservative treatment, status post left rotator cuff repair (2 years ago), status post right hip arthrodesis, obesity, and right knee pain.

Plan: The patient is scheduled to undergo manipulation under anesthesia. The patient is prescribed morphine and duloxetine. The patient is recommended to perform physical therapy 5–6 times per week. The patient is advised to follow up in 2-3 weeks post-procedure. (Pdf 17: p. 66-68)

08/28/2023 - Edward Young, M.D. /Jacksonville Orthopaedic Institute

Subjective: The patient presented for a follow-up for a right knee manipulation status post total knee arthroplasty. The pain is rated as 3/10. The pain is described as intermittent, throbbing, burning, and achy.

Exam: The physical examination of the right knee revealed mild-to-moderate diffuse swelling.

Diagnostic Studies: An x-ray of the right knee was performed and reviewed.

Assessment: Status post manipulation under anesthesia (08/16/2023) following cemented right total knee arthroplasty with Smith & Nephew implants (07/19/2023), history of right shoulder impingement with type III acromion, status post left rotator cuff repair, status post right hip arthrodesis, obesity, and right knee pain.

Plan: The patient is prescribed Lortab. The patient is recommended to perform physical therapy. The patient is advised to follow up in four weeks. (Pdf 17: p. 58-60)

09/05/2023 - Anika Goel, M.D. /EMAS Spine and Brain Specialists.

Subjective: The patient presented for a follow-up with MRI and VNG results. She reported no significant change in her symptoms since the last visit.

Exam: Same as previous.

Assessment: Concussion, mild cognitive impairment, dizziness, headaches, migraine without aura and status migrainosus, and postconcussion syndrome.

Plan: The patient is prescribed nortriptyline, rizatriptan, meclizine, and meloxicam. The patient is instructed not to operate machinery, drive, or work at elevated heights while on these medications, and to avoid alcohol or mixing medications unless explicitly directed. The patient is advised to follow up. (Pdf 9: p.4-7)

10/10/2023 - Mark Emas, M.D. /EMAS Spine and Brain Specialists - Neurological Evaluation Report

Subjective: The patient presents for neurological evaluation following a slip-and-fall on 06/11/2023 at an Embassy Suites, where she slipped in the shower, hit the back of her head, and possibly lost consciousness. Her first memory post-fall was her mother entering the room. The patient has experienced worsening migraine headaches. The patient completed balance training at Brooks but still reports dizziness and requests more Meclizine. The patient also reports concussive symptoms: memory issues, poor concentration, blurry vision, and nausea. The neurocognitive assessment score was 60.

Exam: Rales (wet crackles) noted on auscultation over the lungs. There is mild to moderate tenderness, spasm, and reduced range of motion noted in the lumbosacral and sacroiliac joints.

Assessment: None on files.

Plan: The patient is prescribed Ubrelvy, Qulipta, nortriptyline HCL, Pamelor, and meclizine. A cognitive test is ordered on 10/10/2023. The patient is advised to consult with Douglas Green at Hearing and Balance Center. The patient is also advised to follow up in three weeks. The patient followed up for a total of 11 sessions. The last session was on 11/12/2024. (Pdf 6: p. 200-204, 198-199)

11/07/2023 - 03/15/2024 - David Doward, M.D. /Jacksonville Orthopaedic Institute

Subjective: The patient has a complaint of left-sided sciatica pain.

Exam: There is a limited range of motion in the lumbar spine noted. The results are positive for the Slump test and the straight leg raise test.

Assessment: Lumbar stenosis with neurogenic claudication and lumbar radiculopathy.

Plan: The patient is scheduled to undergo a series of 2 transforaminal epidural steroid injections at the left L5-S1 and S1-S2 levels. The patient is advised to follow up. (Pdf 17: p. 34-37)

11/27/2023 - Edward Young, M.D. /Jacksonville Orthopaedic Institute

Subjective: The patient presented for a follow-up of right knee pain. The pain is rated as a 4/10 and 5/10 at worst. The pain is described as achy, intermittent, activity-related, and occasionally awakens her from sleep.

Exam: The physical examination of the right knee revealed mild-to-moderate diffuse swelling.

Diagnostic Studies: An x-ray of the right knee was performed and reviewed.

Assessment: Status post manipulation under anesthesia following cemented right total knee arthroplasty with Smith & Nephew implants; history of right shoulder impingement with type III acromion; status post left rotator cuff repair; status post right hip arthrodesis; obesity; and right knee pain.

Plan: The patient is recommended to continue low-impact exercise, weight loss, stretching, and strengthening. The patient is advised to follow up in three months. An x-ray of the right knee is ordered on 11/27/2023. (Pdf 17: p. 31-33)

11/28/2023 - David Doward, M.D. /Jacksonville Orthopaedic Institute

Subjective: The patient has a complaint of bilateral lower back pain. The patient presented for left L5-S1, S1-S2 transforaminal epidural steroid injection.

Assessment: Lumbar stenosis with neurogenic claudication and lumbar radiculopathy.

Procedure: Left L5-S1 and S1-S2 transforaminal epidural steroid injection under fluoroscopic guidance.

Plan: None on files. (Pdf 17: p. 23-25)

11/29/2023 - 11/12/2024 - Mark Emas, M.D. /EMAS Spine and Brain Specialists

Subjective: The patient has complaints of cervical spine pain, lumbosacral spine pain, and headache. The cervical spine pain is rated as 6/10. The lumbosacral pain is rated as a 5/10. The headache is rated as 7/10. The pain is described as throbbing, achy, and worsening with activity. The headache frequency is 3-4 times/week, lasting 8–10 hours, moderate to severe. The patient reports daily dizziness, which lasts a few minutes. The left lower extremity dysesthesias/paresthesias/numbness are noted. The concussive symptoms included decreased short-term memory, concentration issues, and dizziness with

movement. The neck pain radiates to the left shoulder. The pain is aggravated by sitting, standing, bending, pushing, pulling, reaching, and lifting. Due to pain, the patient missed a week of work due to neck pain and headaches.

Exam: Rales (wet crackles) noted on auscultation over the lungs. There is mild to moderate tenderness, spasm, and reduced range of motion noted in the lumbosacral and sacroiliac joints.

Assessment: Headache, other symptoms and signs involving cognitive functions and awareness, lumbar back pain, dizziness, and concussion.

Plan: The patient is prescribed Ubrelvy, Qulipta, nortriptyline HCL, lidocaine 5 % topical ointment, and meloxicam. The patient is also prescribed a lumbar back brace. A cognitive test and MRI of the cervical spine were ordered on 10/10/2023. The patient is advised to consult with Douglas Green at Hearing and Balance Center. The patient is scheduled to undergo trigger point injections to the cervical and superior thoracic paraspinal muscles. (Pdf 6: p. 181-185, 176-179)

01/16/2024 - David Doward, M.D. /Jacksonville Orthopaedic Institute

Subjective: The patient has a complaint of lower back pain. The patient presented for left L5-S1, S1-S2 transforaminal epidural steroid injection.

Exam: None on files.

Assessment: Lumbar stenosis with neurogenic claudication and lumbar radiculopathy.

Procedure: Left L5-S1 and S1-S2 transforaminal epidural steroid injection under fluoroscopic guidance.

Plan: None on files. (Pdf 17: p. 16-18)

02/19/2024 - Olga Guskova, PA-C/EMAS Spine and Brain Specialists

Subjective: The patient presented for pain management following a slip and fall on 06/11/2023 at an Embassy Suites hotel, where she slipped in the shower, fell backward, and struck her head. She suspects a brief loss of consciousness. The patient followed up with neurologist Dr. Amy Toenjes for her pre-existing migraines, which worsened after the fall, and was later treated by Dr. Goel with imaging and balance therapy. The patient has complaints of cervical spine pain and lumbosacral pain with associated numbness and tingling in the left upper and lower extremities, radiating pain, and decreased function. The pain is rated as 6/10 and 9/10 at worst. The patient also reports persistent headaches, balance problems, and memory issues. Conservative treatments include chiropractic therapy, trigger point injections by Dr. Emas, and an epidural injection on 01/16/2024. The patient requests additional meclizine for balance issues.

Exam: The patient is in moderate distress. There is facet tenderness, paraspinal muscle spasm, and reduced ROM in all planes noted in the cervical and lumbar spine. The results are positive for the facet loading test, Spurling's test, and lumbar nerve root tension test.

Assessment: Cervical disc herniation, cervical spondylosis, cervical radiculitis, lumbar disc herniation, cervicogenic headache.

Plan: The patient is scheduled to undergo cervical steroid injection at the C7-T1 level. The patient is prescribed hydrocodone-acetaminophen. The patient is advised to follow up in four weeks with Dr. Kramarich. (Pdf 6: p. 151-156)

02/26/2024 - Edward Young, M.D. /Jacksonville Orthopaedic Institute

Subjective: The patient presented for a follow-up of right knee pain. The pain is rated as 2/10 and 4/10 at worst. The pain is described as achy and intermittent.

Exam: The physical examination of the right knee revealed mild-to-moderate diffuse swelling.

Diagnostic Studies: An x-ray of the right knee was performed and reviewed.

Assessment: Status post manipulation under anesthesia following cemented right total knee arthroplasty with Smith & Nephew implants; history of right shoulder impingement with type III acromion; status post left rotator cuff repair; status post right hip arthrodesis; obesity; and right knee pain.

Plan: The patient is advised to follow up in a year. An x-ray of the right knee is ordered on 02/26/2024. (Pdf 17: p. 12-14)

03/15/2024 - David Doward, M.D. /Jacksonville Orthopaedic Institute

Subjective: The patient has complaints of left-sided lower back pain and buttock pain radiating down the left leg to the foot. The patient underwent two left L5-S1 and S1-S2 transforaminal epidural steroid injections previously. The patient completed six weeks of physical therapy. The patient presents with symptoms consistent with neurogenic claudication, which began after a fall in the shower on 06/11/2023 at the Embassy Suites.

Exam: There is a limited range of motion in the lumbar spine, and decreased sensation left L5 and S1 dermatomes. The results are positive for the Slump test and the straight leg raise test.

Assessment: Lumbar radiculopathy and lumbar stenosis with neurogenic claudication.

Plan: The patient is scheduled to undergo a left L5-S1 interlaminar epidural steroid injection. The patient is referred to Dr. Keller. The patient is advised to follow up. (Pdf 17: p. 7-10)

04/01/2024 - Gregory Keller, M.D. /Jacksonville Orthopaedic Institute

Subjective: The patient has a chief complaint of left lower back pain. The lower back pain is rated as 7/10 and 9/10 at worst. The pain is described as throbbing, burning, and achy, occurring intermittently, and radiates from the left lower back down both the posterior and anterior aspects of her left leg to her foot. The patient reports associated numbness, tingling, and a 'needle' sensation in the morning, with her left foot occasionally falling asleep. The symptoms began following a fall in the shower at the Embassy Suites on 06/11/2023, where she landed on her back and hit her head. The patient has undergone chiropractic therapy, and two left L5-S2 and L5-S1 transforaminal epidural steroid injections (TFESIs) on 11/28/2023 and 01/16/2024.

Exam: Normal examination.

Diagnostic Study: An x-ray of the lumbar spine was performed and reviewed.

Assessment: Left-sided sciatic pain due to collapse of L5-S1, causing NF narrowing.

Plan: The patient is scheduled to undergo a left L5 foraminal epidural steroid injection. (Pdf 17: p. 4-6)

07/30/2024 - Stephen Kramarich, M.D. /EMAS Spine and Brain Specialists

Subjective: The patient is presented for a follow-up visit reporting ongoing lower back pain radiating down her left leg, described as a pins-and-needles sensation that disrupts her sleep.

Objective: The physical examination reveals positive findings for sharp/shooting pain and numbness/tingling down the left lower extremity. Limited flexion and extension, tenderness on facet joint palpation, and positive facet loading test with extension and rotation. Spasm and tightness are present.

Assessment: Lumbar radiculitis, cervical disc herniation, cervical radiculitis, cervical spondylosis, cervicogenic headache, lumbar disc herniation.

Plan: The patient is recommended to repeat TFESI at L4-L5 and possibly a CESI at C7/T1. The patient is advised to follow up. The patient is prescribed Norco and Pamelor. (Pdf 6: p. 102-105)

11/12/2024 - Mark Emas, M.D. /EMAS Spine and Brain Specialists

Subjective: The patient has complaints of ongoing post-traumatic symptoms, including cervical spine pain, lumbosacral pain, and headaches located in the occipital region. The cervical spine pain is rated as 6/10. The lumbosacral spine pain is rated as 8/10. The headache is rated as 6/10. The pain is described as constant, throbbing, and achy, and worsens with activities such as sitting, standing, bending, pushing, pulling, reaching, and lifting. The patient also experiences left lower extremity numbness and paresthesia. Additionally, the patient continues to suffer from concussive symptoms, including headaches, dizziness with head or body movement, decreased short-term memory, attention, concentration, and focus. Headaches occur 4–5 times per week, lasting approximately an hour, and range from mild to moderate intensity. Dizziness occurs 2–3 times per week.

Exam: Rales (wet crackles) noted on auscultation over the lungs. There is mild to moderate tenderness, spasm, and trigger points in the cervical and lumbosacral regions.

Assessment: Headache, other symptoms and signs involving cognitive functions and awareness, lumbar back pain, dizziness, and concussion.

Plan: The patient is advised to consult with Douglas Green at Hearing and Balance Center and Lysterly Neurosurgery Group. The patient is referred to a physiotherapist and a vestibular rehab specialist. The patient is recommended to perform physical therapy and chiropractic therapy. The patient is prescribed a custom-fitted cervical therapy collar and lumbar brace. Brain DTI is ordered. The patient is advised to follow up in six weeks. The patient is also prescribed baclofen and memantine. (Pdf 6: p. 1-10)

04/09/2025 - Dashmire Ramadani, M.S., BCBA/River Shores Counseling - Final Evaluation & Findings Report

Subjective: The patient reported that on 06/11/2023, she slipped and fell in a hotel bathtub after water and debris came up from the drain. She stated that she hit her head and passed out until her mother found her, and she sustained a traumatic brain injury. She reported symptoms of anxiety, depression, anger, migraines, pain, and sleep disturbances. Anxiety included fear of falling, avoiding bathtubs and water, anxiety attacks, and sleep disturbances. Depression included crying spells, withdrawal from family and friends, low motivation, and isolation. She reported anger due to pain and forgetfulness since the incident. Migraines were often severe enough to cause her to miss work.

Exam: The patient was assessed with the Adaptive Behavior Assessment System, Third Edition (ABAS-3). She scored in the extremely low category in Leisure, and in the lower part of the average scale for Social, Home Living, and Self-Care. She reported flashbacks, fear of getting hurt, avoidance of water, sleep disturbance, decreased quality of life, and forgetfulness, impacting work and personal life.

Assessment: None on files.

Plan: The patient was recommended to continue ABA Therapy once per week for 6–8 months. Therapy goals included adaptive coping skills, replacement behaviors, engagement, communication, emotional regulation, self-monitoring, and exposure. (Addl 1: Pdf 1: p. 1-3)

**Diagnostic Studies:**

| Date       | Provider/Location                                    | Diagnostic Studies                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
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| 06/13/2023 | James McClellan, M.D./HCA Florida Memorial Hospital  | <p>CT Scan Report of the Brain</p> <p><b>Findings:</b></p> <ul style="list-style-type: none"> <li>The brain parenchyma is normal in appearance without focal lesion, hemorrhage, edema, mass effect, or midline shift. The ventricles and sulci are of normal caliber. There is no extra-axial mass or fluid collection.</li> <li>The visualized portions of the orbits appear unremarkable. Layering fluid is seen within the posterior right maxillary sinus. The remaining visualized paranasal sinuses and mastoid air cells are clear. The osseous structures appear intact.</li> </ul> <p><b>Impressions:</b></p> <ul style="list-style-type: none"> <li>No CT evidence of acute intracranial pathology.</li> <li>Other findings as above. (Pdf 11: p. 32-33)</li> </ul> |
| 06/13/2023 | Jeffrey West, M.D./HCA Florida Memorial Hospital     | <p>X-ray Report of the Right Knee</p> <p><b>Findings:</b></p> <ul style="list-style-type: none"> <li>4 views of the right knee show severe tricompartmental osteoarthritis. There is loss of joint space and bone-on-bone osteoarthritic changes within the medial and patellofemoral compartments. Prominent hypertrophic spurring is present in all 3 compartments. No fracture. No joint effusion. Bone density is normal. Surrounding soft tissues are within normal limits.</li> </ul> <p><b>Impression:</b></p> <ul style="list-style-type: none"> <li>Severe tricompartmental osteoarthritis without acute fracture. (Pdf 11: p. 34)</li> </ul>                                                                                                                         |
| 06/13/2023 | Sean Kuyper, M.D./HCA Florida Memorial Hospital      | <p>X-ray Report of the Lumbar Spine</p> <p><b>Findings:</b></p> <ul style="list-style-type: none"> <li>Anatomic alignment.</li> <li>Moderate L4-5, L5-S1 disc height loss.</li> <li>The facet joints are intact.</li> <li>Mineralization is normal.</li> </ul> <p><b>Impression:</b></p> <ul style="list-style-type: none"> <li>Moderate lower lumbar degenerative disc disease. (Pdf 11: p. 35)</li> </ul>                                                                                                                                                                                                                                                                                                                                                                    |
| 07/19/2023 | Bryan Howze, M.D./Jacksonville Orthopaedic Institute | <p>X-ray Report of the Right Knee</p> <p><b>Findings:</b></p> <ul style="list-style-type: none"> <li>Osseous: No acute fracture or malalignment.</li> <li>Joints: No dislocation. Postsurgical changes from right total knee arthroplasty. Surgical drain in the patellofemoral space.</li> <li>Soft Tissues: Mild soft tissue edema is present. Skin staples noted anteriorly.</li> </ul> <p><b>Impression:</b></p> <ul style="list-style-type: none"> <li>Expected postoperative appearance of the right knee</li> </ul>                                                                                                                                                                                                                                                     |

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|            |                                                       | following arthroplasty. (Pdf 17: p. 82)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
| 08/02/2023 | Richard Newman, M.D./EMAS Spine and Brain Specialists | <p>Videonystagmography Report</p> <p><b>Impressions:</b></p> <ul style="list-style-type: none"> <li>• Peripheral: There is evidence of significant peripheral vestibular dysfunction. Unilateral weakness denotes a lesion involving the horizontal semicircular canal or its afferent pathways on the side of the weaker response.</li> <li>• Central: There is evidence of significant central vestibular dysfunction. Delayed saccades denote a lesion of the frontal or frontoparietal cortex or basal ganglia. (Pdf 9: p. 10-11)</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| 08/23/2023 | Thomas Chu, M.D. /Insite Radiology                    | <p>MRI Report of the Brain</p> <p><b>Findings:</b></p> <ul style="list-style-type: none"> <li>• There are no foci of restricted diffusion to suggest acute ischemic infarction. There are no foci of susceptibility on T2 star sequence to suggest hemorrhage.</li> <li>• The brain is normal in signal intensity and morphology. The ventricles, cisterns and sulci are age-appropriate. There is no parenchymal atrophy. The major intracranial flow voids are unremarkable.</li> <li>• No midline shift, herniation, or hydrocephalus is present. No extra-axial fluid collection, intra-axial mass, or hemorrhage is identified.</li> <li>• The globes and orbits are unremarkable. Mucosal thickening within the left maxillary sinus and ethmoid air cells. Bones and soft tissues are unremarkable.</li> <li>• Incidental note is made of a prominent vessel within the left periventricular white matter. There is otherwise no abnormal enhancement within the brain.</li> </ul> <p><b>Impressions:</b></p> <ul style="list-style-type: none"> <li>• No acute intracranial hemorrhage, infarct, mass effect, or midline shift.</li> <li>• No parenchymal atrophy.</li> <li>• Incidental note is made of a prominent vessel within the left periventricular white matter. There is otherwise no abnormal enhancement within the brain.</li> <li>• Mucosal thickening within the left maxillary sinus and ethmoid air cells. (Pdf 9: p. 8-9)</li> </ul> |
| 09/07/2023 | Shahid M. Nasir, M.D. /Monument-9A Imaging Center     | <p>• MRI Report of the Lumbar Spine</p> <p><b>Findings:</b></p> <ul style="list-style-type: none"> <li>• Mild to moderate lumbar levoscoliosis. No evidence of acute fracture or dislocation. No suspicious focal bony lesion. Conus terminates at L2. No abnormal</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |

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|  |  | <p>cord signal. Mild dependent edema in the posterior paraspinous superficial soft tissues. Paraspinous musculature appears unremarkable. Visualized intra-abdominal contents are within normal limits. Disc desiccation at all lumbar disc levels.</p> <ul style="list-style-type: none"> <li>• T12-L1: No significant disc protrusion, canal stenosis, or foraminal stenosis.</li> <li>• L1-L2: 2-3 mm broad-based disc protrusion, eccentric to the left foramen, causes minimal AP canal stenosis, no significant right foraminal stenosis, and mild left foraminal stenosis.</li> <li>• L2-L3: 2 mm broad-based disc protrusion, eccentric to the left, measures up to 5 mm in the left foramen, causes mild AP canal stenosis, minimal right foraminal stenosis, and moderate left foraminal stenosis and equivocal contact with the left L2-L3 exiting nerve root.</li> <li>• L3-L4: 4-5 mm broad-based disc protrusion, causes moderate AP canal stenosis and moderate bilateral foraminal stenosis. Epidural lipomatosis, contributes to AP canal stenosis. Mild ligamentous hypertrophy. Mild bilateral facet arthrosis.</li> <li>• L4-L5: 4-5 mm broad-based disc protrusion, causes moderate AP canal stenosis and contact and impression on the bilateral L5-S1 nerve roots at this level and causes moderate bilateral foraminal stenosis. Mild to moderate bilateral facet arthrosis.</li> <li>• L5-S1: Mild to moderate anterior osteophytosis. Moderate loss of disc space. Moderate type 2 endplate Modic changes. 2 mm retrolisthesis. 4-5 mm broad-based disc protrusion, eccentric to the left, causes mild AP canal stenosis and contact with the right S1-S2 nerve root at this level and contact and impression on the left S1-S2 nerve root at this level, and causes moderate right foraminal stenosis and contact with the right L5-S1 exiting nerve root, and moderate to marked left foraminal stenosis and contact and impression on the left L5-S1 exiting nerve root. Posterior endplate osseous spurring in the left lateral recess and left foramen, contributes to stenosis. Moderate right facet arthrosis. Moderate to marked left facet arthrosis, contributes to left foraminal stenosis.</li> </ul> <p><b>Impressions:</b></p> <ul style="list-style-type: none"> <li>• Mild to moderate lumbar levoscoliosis.</li> <li>• 2-3 mm broad-based disc protrusion at L1-L2, eccentric to the left foramen, causes minimal AP canal stenosis, no significant right foraminal stenosis, and mild left foraminal stenosis.</li> </ul> |
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|            |                                                    | <ul style="list-style-type: none"> <li>• 2 mm broad-based disc protrusion at L2-L3, eccentric to the left, measures up to 5 mm in the left foramen, causes mild AP canal stenosis, minimal right foraminal stenosis, and moderate left foraminal stenosis and equivocal contact with the left L2-L3 exiting nerve root.</li> <li>• 4-5 mm broad-based disc protrusion at L3-L4, causes moderate AP canal stenosis and moderate bilateral foraminal stenosis. Epidural lipomatosis, contributes to AP canal stenosis. Mild ligamentous hypertrophy. Mild bilateral facet arthrosis.</li> <li>• 4-5 mm broad-based disc protrusion at L4-L5, causes moderate AP canal stenosis and contact and impression on the bilateral L5-S1 nerve roots at this level and causes moderate bilateral foraminal stenosis. Mild to moderate bilateral facet arthrosis.</li> <li>• 2 mm retrolisthesis. 4-5 mm broad-based disc protrusion at L5-S1, eccentric to the left, causes mild AP canal stenosis and contact with the right S1-S2 nerve root at this level and contact and impression on the left S1-S2 nerve root at this level, and causes moderate right foraminal stenosis and contact with the right L5-S1 exiting nerve root, and moderate to marked left foraminal stenosis and contact and impression on the left L5-S1 exiting nerve root. Posterior endplate osseous spurring in the left lateral recess and left foramen, contributes to stenosis. Moderate right facet arthrosis. Moderate to marked left facet arthrosis, contributes to left foraminal stenosis. (Pdf 2: p. 44-45)</li> </ul> |
| 10/10/2023 | EMAS Spine and Brain Specialists                   | <p>Braincheck Report</p> <ul style="list-style-type: none"> <li>• 1st population percentile: Very low. (Pdf 6: p. 195-197)</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| 12/18/2023 | John Hegarty III, M.D. /Monument-9A Imaging Center | <p>MRI Report of the Cervical Spine</p> <p><b>Findings:</b></p> <ul style="list-style-type: none"> <li>• The cervical cranial junction is intact. The right cerebellar tonsil is minimally low lying extending about 0.4 cm inferior to the foramen magnum; the left cerebellar tonsillar is normally located. Straightening of the cervical lordosis suggests muscle spasm. Multilevel mild degenerative disc signal. Mild loss of disc height at C6-C7. Signal in the visualized spinal cord is within normal limits. A 0.7 cm T1 slightly hyperintense to gray matter and CSF pineal region mass is related to the superior aspect of the tectal plate. To the extent visualized, the pituitary appears to be relatively diminutive with a sagittal diameter of about 0.3 cm and this could represent a partially empty sella, which can be an</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |

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|            |                                  | <p>incidental finding or be associated with idiopathic intracranial hypertension.</p> <ul style="list-style-type: none"> <li>• C2-C3: Minimal broad-based disc bulge encroaching upon the anterior thecal sac. The AP diameter of the central spinal canal is not significantly narrowed. No significant foraminal narrowing.</li> <li>• C3-C4: Minimal broad-based disc bulge just touching the anterior surface of the spinal cord as the patient is positioned; the AP central spinal canal diameter is not significantly narrowed. Mild right and mild/moderate left neural foraminal narrowing.</li> <li>• C4-C5: Minimal broad-based disc bulge touching the anterior surface of spinal cord as the patient is positioned; the AP central spinal canal diameter is not significantly narrowed. Minimal left foraminal narrowing suggested.</li> <li>• C5-C6: Minimal/mild broad-based diffuse disc bulge touching the anterior surface of the spinal cord as the patient is positioned; the AP central spinal canal diameter is not significantly narrowed. Suggestion of minimal left foraminal narrowing.</li> <li>• C6-C7: Minimal/mild diffuse disc bulge with a superimposed component of a midline and left paramedian mild broad disc herniation, minimally deforming the anterior left paramedian aspect of the spinal cord, 30/36. Suggestion minimal inferior focal left paramedian subligamentous extension of the disc herniation extending about 0.3 cm inferior to the level of the disc space. Suggestion of bilateral moderate (marked) neural foraminal narrowing.</li> <li>• C7-T1: Minimal broad left paramedian disc herniation minimally deforming the anterior left paramedian aspect of the thecal sac. Suggestion of minimal right foraminal narrowing.</li> </ul> <p><b>Impressions:</b></p> <ul style="list-style-type: none"> <li>• Straightening of the cervical lordosis suggests muscle spasm.</li> <li>• Multilevel generally mild degenerative disc disease, most prominent at C6-C7.</li> <li>• Mildly low-lying right cerebellar tonsil. Neurosurgical consultation might be helpful as indicated.</li> <li>• Findings suggesting a partially empty sella; findings suggesting a small 0.8 cm pineal cyst: Recommend further characterization with an IV contrast brain MRI. (Pdf 2: p. 40-41)</li> </ul> |
| 01/24/2024 | EMAS Spine and Brain Specialists | <p>Braincheck Report</p> <ul style="list-style-type: none"> <li>• 6th population percentile: Low. (Pdf 6: p. 157-160)</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |

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| 05/23/2024 | Shahid M. Nasir, M.D. /Monument-9A Imaging Center | <p>Ultrasound Report of the Neck</p> <p><b>Findings:</b></p> <ul style="list-style-type: none"> <li>Targeted ultrasound of the bilateral submandibular glands.</li> <li>The right submandibular gland measures 2.8 x 3.5 x 1.6 cm and the left submandibular gland measures 3.2 x 3.8 x 1.7 cm. The submandibular glands are symmetric and normal size. No submandibular gland mass or abnormal vascularity.</li> </ul> <p><b>Impressions:</b></p> <ul style="list-style-type: none"> <li>The submandibular glands appear sonographically unremarkable. If symptoms persist or clinically indicated, CT soft tissue neck without and with intravenous contrast would be helpful for further evaluation. (Pdf 2: p. 39)</li> </ul> |
| 07/22/2024 | EMAS Spine and Brain Specialists                  | <p>Braincheck Report</p> <ul style="list-style-type: none"> <li>5th population percentile: Low. (Pdf 6: p. 106-109)</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| 08/05/2024 | Mark Emas, M.D./EMAS Spine and Brain Specialists  | <p>Electroencephalogram Report</p> <p><b>Interpretations:</b></p> <ul style="list-style-type: none"> <li>Toxic metabolic abnormalities (including medications).</li> <li>Post-traumatic concussive symptoms/mild TBI.</li> <li>Other diffuse or multifocal cerebral changes.</li> <li>No epileptiform activity was seen during this study.</li> <li>If clinically indicated, a cerebral MRI may be beneficial to further evaluate this patient's</li> <li>post-traumatic concussive symptoms. (Pdf 6: p. 97)</li> </ul>                                                                                                                                                                                                           |
| 08/05/2024 | Mark Emas, M.D./EMAS Spine and Brain Specialists  | <p>EMG/NCV Study Report of the Bilateral Lower Extremities</p> <p><b>Impressions:</b></p> <ul style="list-style-type: none"> <li>Evidence consistent with a mild chronic left S1 lumbosacral radiculopathy.</li> <li>There was no evidence of a post-traumatic lower extremity plexopathy.</li> <li>There was no evidence of an entrapment neuropathy or mononeuropathy in the lower extremities. (Pdf 6: p. 98-101)</li> </ul>                                                                                                                                                                                                                                                                                                   |
| 10/21/2024 | Mark Emas, M.D./EMAS Spine and Brain Specialists  | <p>Braincheck Report</p> <ul style="list-style-type: none"> <li>1st population percentile: Very low. (Pdf 6: p. 69-72)</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| 02/01/2025 | Shahid M. Nasir, M.D./Monument-9A Imaging Center  | <ul style="list-style-type: none"> <li>MRI Report of the Lumbar Spine</li> <li><b>Findings:</b></li> <li>Stable mild to moderate lumbar levoscoliosis. No evidence of acute fracture or dislocation. No suspicious focal bony lesion. Conus terminates at L2. No abnormal cord signal. Mild dependent edema in the posterior paraspinous superficial soft tissues. Paraspinous musculature appears</li> </ul>                                                                                                                                                                                                                                                                                                                     |

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|  |  | <p>unremarkable. Visualized intra-abdominal contents are within normal limits. Prior partially imaged 22 x 16 mm mixed T1 signal and T2 hyperintense focus in the left adnexa, is not visualized on today's study, and may have represented a complex ovarian cyst. Disc desiccation at all lumbar disc levels.</p> <ul style="list-style-type: none"> <li>• T11-T12: Partially imaged disc level. Mild anterior osteophytosis. Left foraminal annular tear. 3-4 mm broad-based disc protrusion, eccentric to the left foramen, causes mild AP canal stenosis, no significant right foraminal stenosis, and moderate left foraminal stenosis and equivocal contact with the left T11-T12 exiting nerve root. Findings are essentially unchanged. Dedicated MRI of the thoracic spine would be helpful for further evaluation.</li> <li>• T12-L1: No significant disc protrusion, canal stenosis, or foraminal stenosis.</li> <li>• L1-L2: 2-3 mm broad-based disc protrusion, eccentric to the left foramen, causes minimal AP canal stenosis, no significant right foraminal stenosis, and mild left foraminal stenosis. Findings are essentially unchanged.</li> <li>• L2-L3: 2 mm broad-based disc protrusion, eccentric to the left, measures up to 5 mm in the left foramen, causes mild AP canal stenosis, minimal right foraminal stenosis, and moderate left foraminal stenosis and equivocal contact with the left L2-L3 exiting nerve root. Findings are essentially unchanged.</li> <li>• L3-L4: New small annular tear in the posterior central aspect of the disc. 4-5 mm broad-based disc protrusion, causes moderate AP canal stenosis and moderate bilateral foraminal stenosis. Epidural lipomatosis, contributes to AP canal stenosis. Mild ligamentous hypertrophy, Mild bilateral facet arthrosis. Findings are essentially unchanged.</li> <li>• L4-L5: 1 mm retrolisthesis. 4-5 mm broad-based disc protrusion, causes moderate AP canal stenosis and contact and impression on the bilateral L5-S1 nerve roots at this level and causes moderate bilateral foraminal stenosis. Mild ligamentous hypertrophy. Mild to moderate bilateral facet arthrosis. Findings are essentially unchanged.</li> <li>• L5-S1: Mild to moderate anterior osteophytosis. Moderate loss of disc space. Moderate type 2 endplate Modic changes. 2 mm retrolisthesis. 4-5 mm broad-based disc protrusion, eccentric to the left, causes mild AP canal stenosis and contact with</li> </ul> |
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|--|--|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|  |  | <p>the right S1-S2 nerve root at this level and contact and impression on the left S1-S2 nerve root at this level, and causes moderate right foraminal stenosis and contact with the right L5-S1 exiting nerve root, and moderate to marked left foraminal stenosis and contact and impression on the left L5-S1 exiting nerve root. Posterior endplate osseous spurring in the left lateral recess and left foramen, contributes to stenosis. Moderate right facet arthrosis. Moderate to marked left facet arthrosis, contributes to left foraminal stenosis. Findings are essentially unchanged.</p> <p><b>Impressions:</b></p> <ul style="list-style-type: none"><li>• Left foraminal annular tear at T11-T12. 3-4 mm broad-based disc protrusion at T11-T12, eccentric to the left foramen, causes mild AP canal stenosis, no significant right foraminal stenosis, and moderate left foraminal stenosis and equivocal contact with the left T11-T12 exiting nerve root. Findings are essentially unchanged. Dedicated MRI of the thoracic spine would be helpful for further evaluation.</li><li>• 2-3 mm broad-based disc protrusion at L1-L2, eccentric to the left foramen, causes minimal AP canal stenosis, no significant right foraminal stenosis, and mild left foraminal stenosis. Findings are essentially unchanged.</li><li>• 2 mm broad-based disc protrusion at L2-L3, eccentric to the left, measures up to 5 mm in the left foramen, causes mild AP canal stenosis, minimal right foraminal stenosis, and moderate left foraminal stenosis and equivocal contact with the left L2-L3 exiting nerve root. Findings are essentially unchanged.</li><li>• New small annular tear in the posterior central aspect of the L3-L4 disc. 4-5 mm broad-based disc protrusion at L3-L4, causes moderate AP canal stenosis and moderate bilateral foraminal stenosis. Epidural lipomatosis, contributes to AP canal stenosis. Mild ligamentous hypertrophy. Mild bilateral facet arthrosis. Findings are essentially unchanged.</li><li>• 1 mm retrolisthesis. 4-5 mm broad-based disc protrusion at L4-L5, causes moderate AP canal stenosis and contact and impression on the bilateral L5-S1 nerve roots at this level and causes moderate bilateral foraminal stenosis. Mild ligamentous hypertrophy. Mild to moderate bilateral facet arthrosis. Findings are essentially unchanged.</li></ul> |
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|  |  | <ul style="list-style-type: none"> <li>• 2 mm retrolisthesis. 4-5 mm broad-based disc protrusion at L5-S1, eccentric to the left, causes mild AP canal stenosis and contact with the right S1-S2 nerve root at this level and contact and impression on the left S1-S2 nerve root at this level, and causes moderate right foraminal stenosis and contact with the right L5-S1 exiting nerve root, and moderate to marked left foraminal stenosis and contact and impression on the left L5-S1 exiting nerve root. Posterior endplate osseous spurring in the left lateral recess and left foramen, contributes to stenosis. Moderate right facet arthrosis. Moderate to marked left facet arthrosis, contributes to left foraminal stenosis. Findings are essentially unchanged.</li> <li>• Degenerative change, as noted. (Pdf 2: p. 37-38)</li> </ul> |
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07/13/2023 - Edward Young, M.D. – Reviewer - Jacksonville Orthopaedic Institute – X-ray Report of the Right Knee. **Impressions:** Bone-on-bone appearance in the medial compartment, tricompartmental osteophytes with subchondral sclerosis is noted. (Pdf 17: p. 90)

08/15/2023 - Edward Young, M.D. – Reviewer - Jacksonville Orthopaedic Institute – X-ray Report of the Right Knee. **Impression:** Good alignment of the implants, no progressive radiolucencies. (Pdf 17: p. 67)

08/28/2023 - Edward Young, M.D. /Jacksonville Orthopaedic Institute - X-ray Report of the Right Knee. **Impression:** Good alignment of the implants, no progressive radiolucencies. (Pdf 17: p. 59)

11/27/2023 - Edward Young, M.D. /Jacksonville Orthopaedic Institute - X-ray Report of the Right Knee. **Impressions:** Good alignment of the implants, central tracking of patella noted, no progressive radiolucencies. (Pdf 17: p. 32)

02/26/2024 - Edward Young, M.D. /Jacksonville Orthopaedic Institute - X-ray Report of the Right Knee. **Impressions:** Good alignment of the implants, no progressive radiolucencies. (Pdf 17: p. 13)

04/01/2024 - Gregory Keller, M.D. – Reviewer - Jacksonville Orthopaedic Institute – X-ray Report of the Lumbar Spine. **Impressions:** A right hip fusion with 2 cm LL discrepancy. There are severe degenerative changes L4-L5 and L5-S1 and to lesser degree L3-L4 (Pdf 17: p. 5)

### Procedures:

| Date       | Provider/Location                                      | Procedure                                                                                                                                                      |
|------------|--------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 07/19/2023 | Edward Young, M.D. /Jacksonville Orthopaedic Institute | <ul style="list-style-type: none"> <li>• Cemented right total knee arthroplasty using the Smith and Nephew Journey II system. (Pdf 17: p. 83-87)</li> </ul>    |
| 08/16/2023 | Edward Young, M.D. /Jacksonville Orthopaedic Institute | <ul style="list-style-type: none"> <li>• Right knee manipulation under anesthesia. (Pdf 17: p. 61-62)</li> </ul>                                               |
| 11/28/2023 | David Doward, M.D./Jacksonville Orthopaedic Institute  | <ul style="list-style-type: none"> <li>• Left L5-S1 and S1-S2 transforminal epidural steroid injection under fluoroscopic guidance. (Pdf 17: p. 24)</li> </ul> |
| 11/29/2023 | Mark Emas, M.D. /EMAS Spine and                        | <ul style="list-style-type: none"> <li>• Cervical trigger point injections to the right trapezius,</li> </ul>                                                  |

|            |                                                       |                                                                                                                                                                                            |
|------------|-------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|            | Brain Specialists                                     | left levator scapulae, and left and right semispinalis muscles. (Pdf 6: p. 188)                                                                                                            |
| 01/16/2024 | David Doward, M.D./Jacksonville Orthopaedic Institute | <ul style="list-style-type: none"> <li>Left L5-S1 and S1-S2 transforminal epidural steroid injection under fluoroscopic guidance. (Pdf 17: p. 17)</li> </ul>                               |
| 10/21/2024 | Mark Emas, M.D. /EMAS Spine and Brain Specialists     | <ul style="list-style-type: none"> <li>Cervical trigger point injections to the right trapezius, left levator scapulae, and left and right semispinalis muscles. (Pdf 6: p. 77)</li> </ul> |

## Complaints:

As of 8/18/25, Ms. Xxxx complains of:

#Headaches- She states she has migraine headaches at least 4 days per week. She has an aura of stars and double vision. She takes her medication when they begin to occur. She will take Ubrevly. She needs 2-3 doses for it to stop. She goes into a dark room. Light makes her headaches worse. She maintains a quiet environment with low lights most the time to help prevent headaches.

#Muscle Spasms of left leg and left calf- She rates it as 8 out of 10 pain. She must stand up, walk around, and massage her leg to reduce the pain. She has had these since the fall. She has medications which helps a little bit. She cannot walk around the block because she will develop a muscle spasm. Methocarbamol 500mg has helped in the past to reduce the spasms.

#Insomnia- She has trouble falling and staying asleep. She has nightmares that are usually about water. She is also afraid to fall asleep because of fear of waking up from a nightmare. She is seeing a psychiatrist now and started medications that include nortriptyline 50mg, hydroxyzine 25mg, and prazosin 7 mg. She had a recent home sleep study on August 5, 2025, and was told that a C-PAP machine would be ordered based on the sleep study results. She has a follow up appointment in October 2025 regarding her sleep. She never had sleep problems before the fall. She is tired and not refreshed in the morning.

#Dizzy- Every morning when she wakes up, she is dizzy. The room appears to be spinning. She waits to get out of bed until the dizziness subsides. She then gets out of bed and then feels nauseated. It takes her time to get her bearings upon standing. She then holds onto the walls in her hallway. Her father made her a cane that she uses to help her maintain balance.

#Nausea- This is slowly resolving. She used to vomit every day. She now vomits about once every three to four days.

#Vision Impairment- She reports having Blurry Vision. She will see double and triple images. She has double vision when she works on her computer. She needs to close her eyes and readjust her eyes. She has light sensitivity. She has seen an optometrist, Muneer Hanna OD, who prescribed reading glasses. Her vision worsened since the accident, but she wore reading glasses before, now the prescription is stronger.

#S/P Lumbar Spine surgery- She relays this surgery was due to her fall. She fell and hit her posterior head and low back. She reports having decreased flexibility since her surgery. She cannot do what she used to do. She had a home health care nurse after the surgery, but she did not have any physical therapy after the surgery. She does not have low back pain, just limited flexibility. She does perform a home exercise program.

#Neck Pain- She rates this as 6 out of 10 pain. It is a very sharp pain. Her neck is very stiff until she starts to move it around. She reports tingling that goes down her left arm to her left third and fourth finger. She reports

tingling up the back of her head. Dr Emas has provided occipital nerve blocks. The blocks help for about 1-2 days and then it wears off.

#Cognitive Impairment- She is constantly writing reminders down, making sticky notes, and is currently working from home due to her cognitive impairment. She takes Memantine 10mg bid and feels her memory is getting better since she started the medication. She has not seen a speech therapist nor received any type of cognitive therapy, but she says Dr Emas wants to refer her to such.

#Mood Lability- She reports anxiety and takes medication for it. She gets upset when she forgets things. She states she never thought her conditions would go on for so long and that this makes her upset. Her entire life has changed because of this incident. She will not go outside if it is raining. She can take a shower and/or bath, but she gets anxiety. She feels anxiety with nearly anything related to water.

#Right Knee Replacement- This was scheduled prior to fall.

### **Treatment:**

Home exercise program (HEP).

Cemented right total knee arthroplasty using the Smith and Nephew Journey II system, right knee manipulation under anesthesia, cervical trigger point injections to the right trapezius, left levator scapulae, left and right semispinalis muscles, cervical trigger point injections to the right trapezius, left levator scapulae, left and right semispinalis muscles, left L5-S1 and S1-S2 transforaminal epidural steroid injection under fluoroscopic guidance, left L5-S1 and S1-S2 transforaminal epidural steroid injection under fluoroscopic guidance.

-Ms. Xxxx states, "I am doing things to minimize nightmares and prevent migraines. I had seen a counselor and see a psychiatrist once a month. I wear a lumbar back brace and neck brace basically almost non-stop, except for the 3-4 hours which I take off and then put it back on. I currently take ubrevly, nortriptyline, methocarbamol, baclofen, prazosin, topiramate, memantine, hydroxyzine. I did not take these medications before the slip and fall. "

### **Previous Medical History:**

10/21/2005 - Edward Young, M.D. - Procedure Report. Intra-articular injection of Duramorph and Marcaine into the knee joint. (Pdf 17: p. 169)

04/27/2012 - Carole L. Neuman, M.D. /Memorial Hospital Jacksonville - History and Physical Note. Subjective: The patient presented with a complaint of uterine fibroids. Plan: The patient is scheduled to undergo a total abdominal hysterectomy. (Pdf 16: p. 15-16)

04/27/2012 - Carole L. Neuman, M.D. /Memorial Hospital Jacksonville – Operative Note. Procedure - Supracervical and abdominal hysterectomy. (Pdf 16: p. 76-77)



04/27/2012 - Carole L. Neuman, M.D. /Memorial Hospital Jacksonville - Discharge Summary Report. Subjective: The patient is discharged. Exam: The physical examination revealed multiple fibroids, and the largest measured about 6 cm. Assessment: Uterine fibroids. Plan: The patient is advised to follow up in two weeks. (Pdf 16: p. 12-13)

01/27/2013 - Todd Boring, M.D. /Memorial Hospital Jacksonville - Emergency Department Report. Subjective: The patient presented in the emergency department with complaints of chest pain. The pain is described as moderate. The patient stated pain feels like something is kicking her. The pain is exacerbated by deep breathing. Assessment: Chest pain. Plan: The patient is prescribed aluminum hydroxide/magnesium hydroxide, Simethicone, Aspirin, Belladonna alkaloids/phenobarbital, Lidocaine, and ketorolac tromethamine. Condition: Stable. Disposition: Admit. (Pdf 16: p. 290-298)

01/27/2013 - Nam Phuong, D.O. /Memorial Hospital Jacksonville - History and Physical Note. Subjective: The patient presented for history and physical examination with the complaints of sudden onset of sharp left-sided chest pain and shortness of breath that awakened her from sleep. The pain worsened with deep breaths, and she experienced gasping. Assessment: Chest pain, obesity, and migraines. Plan: The patient is advised to follow up and is suggested for a stress test. (Pdf 16: p. 308-309, 310-311)

01/27/2013 - Abdul R. Kani, M.D. /Memorial Hospital Jacksonville - Consultation Note. Subjective: The patient presented for an evaluation with the complaints of stress and lack of sleep due to overwork as possible contributing factors. Assessment: Chest pain, obesity, possible stress and anxiety, and migraines. Plan: Lab test is ordered. The patient is advised to follow up with Dr. S. Patel in 1–2 weeks. (Pdf 16: p. 312-314, 315)

01/27/2013 - Jay E. Xxxx, M.D. /Memorial Hospital Jacksonville - X-ray Report of the Chest. **Findings:** Frontal view of the chest is within normal limits. The heart, mediastinum, lungs, and bony thorax are normal. **Impression:** Normal chest x-ray. (Pdf 16: p. 358)

01/27/2013 - Steven B. Kailes, M.D. /Memorial Hospital Jacksonville - ECG Study Report. **Impressions:** Normal sinus rhythm. Normal ECG. (Pdf 16: p. 361, 363)

01/27/2013 - Lawrence Kanter, M.D. /Memorial Hospital Jacksonville - ECG Study Report. **Impressions:** Normal sinus rhythm. Normal ECG. (Pdf 16: p. 362, 364)

01/28/2013 - Salil Patel, M.D. /Memorial Hospital Jacksonville - Nuclear Exercise Stress Test Report. **Impressions:** 1.) Average exercise capacity with no ischemic ECG changes. 2.) Normal perfusion and normal left ventricular function. 3.) Low-risk study. (Pdf 16: p. 365-366)

01/28/2013, Salil Patel, M.D. /Memorial Hospital Jacksonville - Treadmill Exercise Test. **Impressions:** Normal sinus rhythm with no ST-segment changes. (Pdf 16: p. 359-360)

01/28/2013 - Nam Phuong T. Lu, D.O. /Memorial Hospital Jacksonville - Discharge Summary Report. Subjective: The patient is discharged. Assessment: Obesity, Migraine, chest pain, possibly musculoskeletal. Plan: The patient is advised to follow up with Dr. Ortiz-Roldan. (Pdf 16: p. 284-285)

02/16/2014 - Michael L. Cordova, M.D./Memorial Hospital Jacksonville -Emergency Department Report. Subjective: The patient presented to the Emergency Department with complaints of mild achy back and right knee pain. The symptoms have been persistent but mild. Exam: Mild tenderness over the right anterior knee and mild midline lumbar tenderness, and right-sided paraspinal muscle tenderness. Assessment: Back pain and knee contusion. Plan: The patient is advised to follow up. Condition: Stable. Disposition: Home. (Pdf 16: p. 434-439)

02/16/2014 - Scott Kaltman, M.D./Memorial Hospital Jacksonville - X-ray Report of the Right Knee. **Findings:** There is no evidence of suprapatellar joint evidence for acute fracture. No dislocation compartment right knee osteoarthritic change space narrowing, degenerate spurring. **Impression:** Three-compartment right knee osteoarthritic changes. (Pdf 16: p. 449)

02/16/2014 - Andrew McBride, M.D./Memorial Hospital Jacksonville - X-ray Report of the Lumbar Spine. **Findings:** Extensive postsurgical changes are seen within the pelvis. There is normal alignment. No evidence of spondylolisthesis. There is joint space narrowing of L4-L5 without an acute osseous abnormality. **Impression:** Extensive surgical hardware within the pelvis without evidence of acute osseous abnormality of the lumbar spine. (Pdf 16: p. 450)

02/25/2014 - Edward Young, M.D. /Jacksonville Orthopaedic Institute - Evaluation Report. Subjective: The patient presented for an evaluation with the complaints of left shoulder pain, lower back pain, and right knee pain. The left shoulder pain developed in a delayed fashion, while the back and knee pain have been persistent since the accident. Exam: The physical exam revealed mild joint effusion and point tenderness over the right medial knee joint line, with a positive medial Apley grind test, mild patellofemoral crepitus, and tenderness. Tenderness at the lumbosacral junction with mild to moderate paraspinal muscle spasm. Positive Hawkins, Neer, Empty Can, and markedly positive O'Brien's sign. Assessment: Lumbar strain; exacerbation of underlying posttraumatic arthritis, right knee; rule out rotator cuff tear, left shoulder. Plan: MRI of the left shoulder ordered. The patient is prescribed ibuprofen and Flexeril. The patient followed up for a total of 11 sessions. The last session is on 05/02/2023. (Pdf 17: p. 165-168)

02/26/2014 - Joshua Yellin, M.D. /Precision Imaging Centers -MRI Report of the Left Shoulder. **Findings:** A subtle contour deformity is present to the posterior aspect of the humeral head and may represent an old Hill-Sachs lesion. Supraspinatus tendinitis is present along with an intermediate-grade longitudinal tear present to the distal supraspinatus tendon which is best seen on image 10 of series 6. A low-grade partial-thickness tear is present to the posterior fibers of the infraspinatus musculotendinous junction and is best seen on image 10 of series 2. Subscapularis and teres minor tendons are intact. Subtle linear signal is seen to the posterior aspect of the glenoid labrum but does not clearly represent a tear. Articular cartilage is intact. Long head biceps tendon is intact. Mild narrowing of the subacromial space with trace subacromial/subdeltoid bursitis. Low-grade partial-thickness interstitial tear to the teres minor muscle belly. Bone marrow signal is unremarkable. Coracoclavicular ligaments are intact. **Impressions:** Supraspinatus and infraspinatus tendinitis. An intermediate-grade longitudinal tear is present to the distal supraspinatus tendon and is best seen on image 10 of series 6. A low-grade partial-thickness tear is present to the posterior fibers of the infraspinatus musculotendinous junction. A tiny interstitial tear is present to the teres minor muscle belly. 2.) Equivocal linear signal is seen to the posterior portion of the glenoid labrum. If an occult labral tear is suspected, consider MR arthrography for further evaluation. 3.) Subtle contour deformity present to the posterior aspect of the humeral head. This could represent an old, shallow Hill-Sachs lesion. Please correlate this to a

remote history of dislocation. 4.) Mild narrowing of the subacromial space with trace subacromial/subdeltoid bursitis. (Pdf 17: p. 158-159)

03/06/2014 - 05/02/2023 - Edward Young, M.D. /Jacksonville Orthopaedic Institute. Subjective: The patient presented for a follow-up evaluation with a complaint of left shoulder pain. The patient reported worsening pain. Exam: The physical examination reveals range of motion is severely limited, with only 30–40 degrees of flexion and abduction achievable. Assessment: Partial thickness rotator cuff tear, left shoulder; traumatic exacerbation of underlying posttraumatic arthritis, right knee; lumbar strain. Plan: The patient is prescribed hydrocodone-acetaminophen. The patient is advised to follow up in three weeks. (Pdf 17: p. 160-162)

03/10/2014 - Barbara Short /Jacksonville Orthopaedic Institute -Initial Evaluation Note. Subjective: The patient presented for an initial evaluation with a complaint of left shoulder impingement syndrome and pain. She reports numbness and tingling along the left bicep. The pain is rated as 10/10. Exam: The physical examination reveals significant tenderness to light touch over the left shoulder and is highly guarded throughout the assessment. Assessment: Shoulder impingement syndrome and left shoulder pain. Plan: The treatment plan included cryotherapy, electrical stimulation, mobilization/manual therapy, moist heat, PROM (Passive Range of Motion), soft tissue mobilization, stretching, strengthening, ultrasound, shoulder stabilization program, rotator cuff program, and scapular stabilization exercises 2 times per week for 8 weeks. The patient followed up for a total of three sessions. The last session is on 03/24/2014. (Pdf 17: p. 152-154)

03/17/2014 - 03/24/2014 - Barbara Short /Jacksonville Orthopaedic Institute. Subjective: The patient reported continued pain with overhead reaching. Exam: The physical examination reveals limited passive range of motion (PROM) in the left shoulder, with flexion to 90°, abduction to 70°, external rotation to 40°, and internal rotation to 40°. Assessment: None on files. Plan: The treatment plan included manual techniques and education, and a moist heat pack. (Pdf 17: p. 147-148)

03/24/2014 - Barbara Short /Jacksonville Orthopaedic Institute. Subjective: The pain is rated as 6/10. Exam: The physical examination reveals the left shoulder's passive range of motion (PROM) remains limited, with flexion and abduction to 90°, external rotation to 45°, and internal rotation significantly restricted to only 40°. Assessment: None on files. Plan: The treatment plan included manual techniques and education, and a moist heat pack. (Pdf 17: p. 143-144)

10/14/2016 - Zwindia Ortiz Roldan, M.D./First Coast Cardiovascular Institute. Subjective: The patient presented for a preoperative clearance for bariatric surgery due to morbid obesity. Assessment: Obesity class III, elevated cholesterol, and status post arthroscopic surgery of the right knee. Plan: The patient is advised to follow up in three months. (Pdf 16: p. 472-474)

10/14/2016 - Midmark Diagnostic Group -ECG Study Report. **Impressions:** Sinus rhythm within normal limits. (Pdf 16: p. 661)

11/07/2016 - Arundhati Rao, M.D.- Followup Note. Subjective: The patient presented for a preoperative evaluation for a planned laparoscopic vertical sleeve gastrectomy. Assessment: Morbid obesity due to excess calories. Plan: The patient is prescribed Lortab Elixir and omeprazole. The patient is advised to follow up in one week. (Pdf 16: p. 470-471)

11/09/2016 - Arundhati Rao, M.D./Memorial Hospital Jacksonville - Operative Note. Laparoscopic vertical sleeve gastrectomy. (Pdf 16: p. 539-540)

11/10/2016 - Arundhati Rao, M.D./Memorial Hospital Jacksonville - Subjective: The patient presented for an evaluation with a complaint of nausea overnight. Assessment: Laparoscopic vertical sleeve gastrectomy. Plan: The patient is recommended to consume clear liquids and ambulate as tolerated. (Pdf 16: p. 483-486)

11/11/2016 - 11/12/2016 - Steven Webb, M.D./Memorial Hospital Jacksonville. Subjective: The patient presented for an evaluation with a complaint of gas pain. Assessment: Laparoscopic vertical sleeve gastrectomy. Plan: The plan is to advance activity and encourage pulmonary toileting. The patient follows up on 11/12/2016. (Pdf 16: p. 481-482)

11/12/2016 - Steven Webb, M.D./Memorial Hospital Jacksonville. Subjective: The patient presented for an evaluation. The patient still has marginal PO intake. Assessment: Laparoscopic vertical sleeve gastrectomy. Plan: The plan is to advance activity and pulmonary toileting. (Pdf 16: p. 478-480)

11/12/2016 - Steven Webb, M.D./Memorial Hospital Jacksonville - Discharge Summary Report. Subjective: The patient is discharged today. Assessment: Morbid obesity. Plan: The patient is prescribed hydrocodone and famotidine. (Pdf 16: p. 467-469)

04/16/2019 - David Estle, M.D. /Precision Imaging Centers - MRI Report of the Right Shoulder. **Findings:** Rotator cuff: The supraspinatus, infraspinatus, teres minor, and subscapularis tendons are intact. Rotator cuff musculature is normal in bulk and signal intensity. Biceps tendon: The Intra-articular portion of the long head biceps tendon is intact; extra-articular portion is located within the intertubercular groove. Labrum and capsular structures: The labrum is intact. Capsular structures are intact. Glenohumeral joint: There is no acute fracture. No focal glenohumeral cartilage abnormality. Normal morphology of the humeral head and glenoid. Acromioclavicular joint and outlet. There is a type III acromion. No subacromial enthesophyte or thickening of the coracoacromial ligament. The AC joint appears unremarkable. No subacromial bursitis. The deltoid muscle is normal in bulk and signal intensity. (Pdf 17: p. 105)

08/26/2021 - Kamal Bohsali, M.D. /Jacksonville Orthopaedic Institute -Initial Evaluation Note. Subjective: The patient presented for an initial evaluation with a complaint of chronic right knee pain. The pain is rated as a 7/10. She reports associated symptoms including popping, loss of motion, numbness, and swelling in the right knee. Exam: The physical examination reveals tenderness to palpation at the medial joint line and crepitus at the patellofemoral articulation. Assessment: Right knee primary osteoarthritis, right patella chondromalacia, right knee pain. (Pdf 17: p. 97-99).

06/30/2022 - 09/01/2022 - Yomari Rodriguez, D.C., Doctor of Chiropractic/Advanced Healthcare Center - Subjective: The patient presented with complaints resulting from a motor vehicle collision on 06/25/2022. He reported constant pain in the lumbar, cervical, and thoracic regions, described as aching, sharp, stiffness, and tightness. The pain radiated into the left shoulder and remained non-radiating in the lumbar and thoracic regions. The pain was aggravated by almost any activity, including movement, and improved with heat packs. He also complained of headaches described as aching and throbbing, aggravated by light and noise. Exam: On examination, the patient exhibited moderate to severe pain and tenderness in the posterior cervical, mid thoracic, and lumbar regions. There was also hypertonicity noted in the right trapezius, lumbar, and left sacroiliac/piriformis areas. The patient had a restricted range of motion in both the cervical and lumbar spines with pain

in all fields of motion. Assessment: The patient was diagnosed with cervicobrachial syndrome, lumbar strain, and post-traumatic headaches. Plan: The patient's treatment plan included physical therapy, myofascial therapy, and chiropractic adjustments. Supportive modalities like heat packs, cold packs, EMS, and manual therapy were recommended to reduce pain and inflammation and promote healing. The patient was advised to follow up with prescribed exercises and attend therapy 2-3 times per week for 4-6 weeks. The patient followed up for a total of 13 sessions. The last session was on 09/01/2022. (Addl 2: Pdf 1: p. 40-43)

07/05/2022 - 07/14/2022 - Jesse Badger, MS, PA-C/Advanced Healthcare Center - Subjective: The patient was involved in a motor vehicle accident on 06/25/2022, where she was seat-belted, and the impact was from the side, causing headache and musculoskeletal pain. She reported an ongoing headache rated 8/10 in severity, which was aggravated by movement and alleviated by rest. Associated symptoms included photophobia, nausea, and insomnia. She also reported pain in the cervical and lumbar spine with burning and tightness radiating into the left upper extremity, rated 6/10. Additionally, she experienced saddle anesthesia and weakness in the lower extremities. Exam: She exhibited a restricted range of motion in the cervical and lumbar spine, with tenderness and spasm. Assessment: The patient was diagnosed with post-traumatic headache, cervical sprain, lumbar sprain, radiculopathy of the cervical spine, and lower back pain. Plan: The patient was scheduled for MRI and EMG/NCV studies to assess the extent of nerve involvement. The patient followed up once on 07/14/2022. (Addl 2: Pdf 1: p. 6-12)

07/14/2022 - Jesse Badger, MS, PA-C/Advanced Healthcare Center - Subjective: The patient reported a recurrence of headaches (HA) despite receiving a Medrol Dose Pack a few days ago. The headaches were not as severe as before, but were still present at a 5/10 pain level. The pain was described as a dull ache with an occasional burning sensation in the cervical and lumbar regions. The patient also complained of weakness in both upper and lower extremities, with some neurological complaints including radiation of pain into the trapezius, shoulders, and arms. Exam: Unchanged. Assessment: The patient was diagnosed with post-traumatic headache, cervical sprain, lumbar sprain, radiculopathy of the cervical spine, and lower back pain. Plan: The patient was advised to continue with physical therapy and chiropractic care. Diagnostic studies, including an EMG/NCV, were recommended to assess nerve damage. The patient was also prescribed pain management options including a joint injection and SPG block, both to be scheduled. Follow-up with the provider was scheduled for the near future, and the patient was instructed to follow up for a neurology consultation as well. (Addl 2: Pdf 1: p. 1-4)

07/19/2022 - 10/04/2022 - F. Carter Wilson, D.C., Doctor of Chiropractic/Advanced Healthcare Center - Subjective: The patient presented for treatment related to an auto accident. She reported symptoms occurring approximately 90% of the time with a pain level of 7/10. Exam: Pain/tenderness to palpation: Posterior cervical (neck) and lumbar (low back). Plan: The patient was treated with manual therapy for the cervical and lumbar areas to reduce pain and hypertonicity, therapeutic exercises to improve cervical and lumbar strength and flexibility, and strapping/taping to facilitate neuromuscular re-education, balance, and posture for standing or sitting activities. The patient followed up for a total of five sessions. The last session was on 10/04/2022. (Addl 2: Pdf 1: p. 37)

08/24/2022 - Soheil Sooudi, M.D./Advanced Healthcare Center - **Findings:** There is degenerative spondylosis with mild disk space narrowing at C6-C7 level. There is straightening of cervical lordosis. The vertebral bodies are intact with normal marrow signal. The cord shows normal signal. There is no cerebellar tonsillar ectopia. There are degenerative changes of atlanto-odontoid joint. There is motion on the axial images. C2-C3 level, there is no spinal or foraminal stenosis. C3-C4 level, there is mild facet joint arthropathy on the right. There is bulging disk indenting the anterior thecal sac. There is mild left-sided bony foraminal narrowing secondary to uncovertebral joint hypertrophy. C4-C5 level, there is bulging disk indenting the anterior thecal sac. There is mild left-sided bony foraminal narrowing secondary to uncovertebral joint

hypertrophy. C5-C6 level, facet joint arthropathy is noted bilaterally. There is bulging disk indenting the anterior thecal sac without canal stenosis. Foramina are patent. C6-C7 level, there is a posterior left paracentral disk herniation with annular fissure indenting the anterior thecal sac encroaching upon the cord with flattening of the cord noted. There is moderate bilateral foraminal narrowing secondary to uncovertebral joint hypertrophy. C7-T1 level, facet joint arthropathy is noted bilaterally. There is no spinal or foraminal stenosis. **Impressions:** Motion on the axial images. Degenerative spondylosis with disk space narrowing at C6-C7 level with multilevel facet joint arthropathy and straightening of cervical lordosis. Posterior left paracentral C6-C7 level disk herniation with annular fissure indenting the anterior thecal sac, encroaching upon the cord with flattening of the cord noted. Posterior disk bulge at C3-C4 through C5-C6 level without canal stenosis. Foraminal narrowing noted at multiple levels described above more pronounced bilaterally at C6-C7 level. (Addl 2: Pdf 1: p. 16-17)

08/24/2022 - Soheil Sooudi, M.D./Advanced Healthcare Center - **Findings:** The patient is status post right hip arthroplasty. There is a cystic lesion noted within the right side of the pelvis which is most likely ovarian. Pelvic sonogram recommended for further evaluation. It measures about 4.5 cm. There is degenerative spondylosis with disk desiccation and disk space narrowing predominantly at L4-L5 and L5-S1 level and to a lesser extent L3-L4 level. There is lumbar spine levoscoliosis. The vertebral bodies are preserved in height. The conus medullaris is at L1-L2 level. There is right psoas muscle atrophy. L5-S1 level, there is grade 1 retrolisthesis. Facet joint arthropathy with hypertrophy noted on the left side. There is bulging disk with marginal osteophyte formation compressing the anterior thecal sac and resulting in moderate-to-severe left-sided foraminal and lateral recess stenosis impinging on the left L5 nerve root within foramen and left S1 nerve root within lateral recess. There is also mild-to-moderate right-sided foraminal narrowing. L4-L5 level, there is grade 1 retrolisthesis. Facet joint arthropathy is noted bilaterally more pronounced on the left side. There is broad-based protruding posterior right paracentral/lateral disk herniation with annular fissure indenting the anterior thecal sac with mild-to-moderate right lateral recess and foraminal narrowing encroaching upon the right L5 and L4 nerve roots, respectively. L3-L4 level, facet joint arthropathy with hypertrophy noted bilaterally. There is broad-based protruding posterior central disk herniation with annular fissure indenting the anterior thecal sac with moderate central canal and lateral recess narrowing. Thecal sac measures 7 mm in AP dimension. There is broad-based right foraminal disk protrusion with mild-to-moderate foraminal narrowing touching the right L3 nerve root. L2-L3 level, facet joint arthropathy with mild hypertrophy noted bilaterally. There is a broad-based protruding left foraminal disk herniation with moderate foraminal stenosis impinging on the left L2 nerve root. There is minimal posterior disk bulge indenting the anterior thecal sac without canal stenosis. L1-L2 level, there is broad-based left foraminal disk protrusion with mild foraminal narrowing. There is no central canal stenosis. **Impressions:** Degenerative spondylosis with disk space narrowing predominantly at L4-L5 and L5-S1 level and to a lesser extent L3-L4 level with levoscoliosis and multilevel facet joint arthropathy. Grade 1 retrolisthesis noted at L4-L5 and L5-S1 level. Disk pathology throughout lumbar spine as described above. Right psoas muscle atrophy. Right hip arthroplasty. Cystic lesion of the right pelvis. A pelvic sonogram could be considered for further evaluation. (Addl 2: Pdf 1: p. 18-19)

09/01/2022 - Yomari Rodriguez, D.C., Doctor of Chiropractic/Advanced Healthcare Center - **Subjective:** The patient presented for treatment related to an auto accident. She reported her symptoms at a level of 5/10 (VAS scale), with a maximum pain rating of 6/10. The patient described mild improvement since the last visit. **Exam:** The patient exhibited pain and tenderness upon palpation of the posterior cervical, mid-thoracic, and lumbar regions. There was mild to moderate muscular tightness in these areas. Spinal restrictions were noted at C3, C4, C5, T5, T6, T7, T8, T9, L3, L4, and L5. **Plan:** The patient received manual therapy (97140) to decrease trigger points and hypertonicity in the cervical, thoracic, and lumbar regions for 15 minutes, followed by therapeutic exercises (97110) for cervical ROM, cervical strength, lumbar ROM, and lumbar strength. EMS and ultrasound therapy were applied to the left trapezius, left scapular/mid thoracic, left lumbar, and right lumbar regions for 15 minutes. Chiropractic manipulative therapy was performed at C3, C4, C5, T5, T6, T7, T8, L4, and L5 with mechanical adjustments. The patient was advised to continue physical therapy as per the treatment plan. (Addl 2: Pdf 1: p. 26)

10/04/2022 - F. Carter Wilson, D.C., Doctor of Chiropractic/Advanced Healthcare Center - Subjective: The patient presented for treatment related to an auto accident. She described persistent symptoms in the same areas as her previous visit. Her pain occurred approximately 85-90% of the time at a level of 6/10, with a worst pain rating of 7. Exam: Unchanged. Plan: The patient received manual therapy for the cervical and lumbar regions to reduce pain, hypertonicity, and spasm. Therapeutic exercises were prescribed to improve strength, flexibility, and endurance. EMS and ultrasound therapy were applied to the neck and lumbar regions for symptom management. (Addl 2: Pdf 1: p. 20)

01/26/2023 - 05/18/2023 - Robert Hurford, M.D./Southeast Orthopedic Specialists - Subjective: The patient presented for an evaluation with a complaint of lower back pain. The pain has been ongoing since June 25th, 2022. She stated she was involved in an MVC when she was at a red light, and the car behind her ran into her. The pain was focused in the lower back with associated numbness, tingling, and weakness down the left leg to the knee. The pain was worse in the morning, prolonged standing, sitting, walking, bending, lifting, and twisting, which significantly hinders activities of daily living. The pain was rated as 7/10. Exam: The physical examination revealed moderate limitation and pain with lumbar spine motion. Both right and left hip passive range of motion showed no pain with flexion and internal rotation. Diagnostic Studies: An x-ray of the lumbar spine was performed, which showed moderate L3-L4 spondylosis; severe L4-L5, L5-S1 spondylosis. Assessment: Lower back pain. Plan: Prescribed Celebrex. Recommended to start physical therapy. The patient followed up twice. The last session was on 05/18/2023. (Addl 2: Pdf 4: p. 20-23)

03/14/2023 - Katherine McLeod, PA-C/Southeast Orthopedic Specialists - Subjective: The patient presented for re-evaluation of her lumbar spine pain after a motor vehicle accident on 06/25/2022. She reported attending physical therapy since 02/13/23 with no improvement, with 10% improvement from Celebrex prescribed at her last visit. The patient indicated pain across the lumbar back with radiation down the lateral aspect of the left lower extremity, associated with numbness and tingling. The patient rated her pain as 5-6/10 today and 8/10 at its worst. Exam: Lumbar spine motion was moderately limited with pain upon movement. Assessment: Lower back pain, lumbar spondylosis, and spondylosis without radiculopathy. Plan: A left L5 TFESI injection was recommended for diagnostic and therapeutic purposes. The patient was advised to complete physical therapy at Brooks and follow up 3 weeks post-injection for evaluation. (Addl 2: Pdf 4: p. 15-17)

04/13/2023 - Isaac Gooding, M.D./Southeast Orthopedic Specialists - Procedure: Lumbar transforaminal epidural steroid injection, to the left L5-S1. (Addl 2: Pdf 4: p. 12-14)

05/02/2023 - Edward Young, M.D. /Jacksonville Orthopaedic Institute. Subjective: The patient is presented for a follow-up evaluation with a complaint of right knee pain. The pain is rated as 10/10. The pain is described as a popping, grinding, stiffness, throbbing, burning, achy, and swelling. The pain is worsened with time. Exam: The physical exam reveals varus deformity of the right knee with weight-bearing, medial joint line tenderness, and crepitus at the patellofemoral joint. Assessment: Right knee pain status post arthrodesis, right hip, and status post left rotator cuff repair surgery, obesity. Plan: The patient is advised to follow up. (Pdf 17: p. 94-96)

05/18/2023 - Robert Hurford, M.D./Southeast Orthopedic Specialists - Subjective: returned for a follow-up after a recent left L5/S1 TFESI performed on 04/13/23. She reported a 50% improvement in pain during the anesthetic phase, followed by an 80% improvement, which has since plateaued. The pain was rated as 3/10 today in the clinic, and 4-5/10 at its most severe. She also reported improved functionality and movement. Ms. Xxxx has been completing a medically directed home exercise program 3 times a week since her last visit on 01/26/23. Exam: Lumbar motion: Moderate limitation with pain.

Assessment: Lower back pain. Plan: Gabapentin 300 mg was prescribed to aid with nerve pain. The patient was scheduled for a follow-up appointment in 6 weeks. (Addl 2: Pdf 4: p. 8-11)

### **Employment:**

Ms. Xxxx works as an analyst at Florida Blue Cross/ Shield. She handles provider contracts.

She was doing this before her fall. It is harder for her to do her work now because of her memory. She must use sticky notes to not forget activities. She has more trouble sitting for long periods of time. She now has to take more frequent breaks every 30 minutes to 1 hour. She has flexibility at work, as she can make up her time if say she needs to take a break due to a migraine or pain exacerbation.

### **Activities of Daily Living (ADLs):**

Ms. Xxxx has ongoing pain in the neck and migraines in addition to post-concussion related sequelae that are associated with a significant physical functional impairment that affects several ADLs.

She lives independently and is able to cook and clean for herself. Her mother will help her do laundry when she bends over, she will have dizziness exacerbations.

It takes her much longer to cut her yard because of dizziness and balance.

She does drive, but if she has blurry vision she will pull over.

It takes her longer to get dressed. She is afraid to take a shower or bath.

### **Financial Barriers to Mental Health and Other Health Care**

Ms. Xxxx reports financial barriers to obtaining medical diagnostic evaluation and treatment.

Specifically, she states that while she has insurance and Florida Blue pays her claim, the copayments have taken a toll on her financially. She has not yet gone to vestibular therapy, for example because of copayment costs being difficult for her to afford.

### **Psychosocial Barriers to Recovery:**

According to the ODG Evidence-Based Medicine (EBM) Medical Treatment Guidelines (MTGs) when a person has an injury that results in both pain and loss of function that lasts for greater than 6 months, and especially if they have had invasive procedures, or are not improving as expected, they likely to develop psychosocial barriers to recovery that can significantly impact an individual's recovery and rehabilitation process. Here are some key psychosocial barriers to consider.

*Depression and anxiety* are common psychological barriers that can hinder recovery and are also associated with insomnia and poor sleep patterns.

*Fear* of pain or re-injury can lead to avoidance behaviors that slow recovery.



*Catastrophic thinking*, where individuals expect the worst outcomes, can also impede progress.

*Avoidance* and fear behaviors develop where individuals don't obtain the surgery or injections when recommended by their physicians.

*Stigma* associated with mental health issues or certain injuries can prevent individuals from seeking necessary help or fully engaging in treatment.

These barriers would need to be evaluated by a *psychologist* trained in the treatment of chronic injury and pain and require a referral from a treating physician.

These barriers are addressed with short courses (4-6 sessions) of supportive psychotherapy during active treatment, often followed by a course of functional rehabilitation when the individual reaches a medical plateau.

Ms. Xxxx has had ongoing headache, neck pain, and low back stiffness pain for 27 months. All of the documented treatment for the injury has been entirely focused on Ms. Xxxx's initial presenting physical injuries. These physical conditions are associated with easy aggravation with regular activities and secondary loss of function. Ms. Xxxx has had trigger point injections, occipital nerve blocks, epidural injections, and lumbar spine surgery. Ms. Xxxx now has the secondary onset of symptoms of anxiety, depression, insomnia and fear/avoidance behaviors that likely represent psychosocial barriers to her physical and functional recovery. Ms. Xxxx is expected to benefit from a course of supportive psychotherapy during her course of physically oriented treatment.

After completing all of the necessary physically oriented treatment and supportive psychotherapy, Ms. Xxxx will require a functional restoration program to teach Ms. Xxxx pain-coping skills, prevention of aggravations, a lifelong rehabilitative exercise program and substitute pain therapies other than medications and injections.

### **Previous Medical/Surgical History:**

Medical: Migraines, anemia, and osteoarthritis. (Pdf 17: p. 88). She had headaches before the fall that were under control. After the fall, the migraines were exacerbated. Headaches were one side of her head but now also present on the posterior side of her head.

Surgery: Hysterectomy-partial (08/26/2021), right knee scope (08/26/2021), right knee arthroscopy and microfracture (04/11/2019), rotator cuff repair (04/11/2019). (Pdf 17: p. 88), prior right hip surgery at age 12

Low back surgery March 13, 2025- 3 esi prior. Epidural steroid injections did not take away her pain.

### **Interview with Ms. Xxxx**

Ms. Xxxx was interviewed by Dr. Glazer on 08/18/25 for 50 minutes. The information obtained from this interview is provided throughout this report.

### Peer-to-Peer Conversation:

Dr. Glazer had a peer-to-peer conversation with Dr. Mark Emas, neurology on 09/08/2025 for 20 minutes. The discussion revealed that Dr. Emas last saw the injured party on 07/14/2025. At that time, she reported memory impairment, neck pain, low back pain, lower extremity radicular pain, headaches, vestibular impairment all related to the fall. Dr Emas believes that all of the evaluation and treatment documented above is related to the slip and fall of 06/11/2023. He also recommended two to three visits with neurology per year, medications to include gabapentin, pamelor, ubrevly, memantine, and baclofen, blood lab work at least twice per year, MRI every 5 years of the Brain, Cervical Spine and Lumbar spine, one VNG every 4 years, one EMG/NCS every 4 years of the lower extremities, complex cognitive evaluation every 3-5 years with a neuropsychologist, referral to vestibular therapy with 4-6 visits every 3 years. Dr Emas noted that the injured party will continue to have repeated flare-ups two times a year and that these flare-ups would increase in frequency as the individual gets older, requiring physician re-evaluation, medication management and therapy. Dr. Emas noted that the individual had an injury that has resulted in pain and loss of function for 27 months and is likely to have developed psychosocial barriers to recovery at this time that would need to be evaluated by a psychologist. Dr Emas notes that a TENS device would be beneficial for long term pain management and to reduce the use of medications. Dr Emas also recommends referral to pain management. Dr. Emas states that she has Blue Cross insurance coverage and any financial barriers could come from any such copays and deductibles.

### Life Expectancy:

According to the US Social Security Administration, (<https://www.ssa.gov/OACT/population/longevity.html>), based on the age and sex of Ms. Xxxx the average life expectancy of Ms. Xxxx is 85.7 years with 32.2 more years of life. Many other databases are available to predict life expectancy, however, the US Social Security Administration database is designed to specifically predict an **individual's personal longevity**, and is by far the most robust statistically with many decades of continual data collection and hundreds of millions of US individuals having a fair representation of the common risk factors affecting longevity, such as hypertension, smoking cigarettes, obesity, and diabetes mellitus type 2.

### Impression:

Based on the clinical records and information provided above, Ms. Xxxx has the following diagnoses and associated impairment associated with the injury:

**#Mild Traumatic brain injury with related sequelae to include insomnia, migraine, dizziness, nausea, vision impairment, cognitive impairment, and mood lability.**

**#Cervical Spondylosis due to MRI findings of:**

- The cervical cranial junction is intact. The right cerebellar tonsil is minimally low lying extending about 0.4 cm inferior to the foramen magnum; the left cerebellar tonsillar is normally located. Straightening of the cervical lordosis suggests muscle spasm. Multilevel mild degenerative disc signal. Mild loss of disc height at C6-C7. Signal in the visualized spinal cord is within normal limits. A 0.7 cm T1 slightly hyperintense to gray matter and CSF pineal region mass is related to the superior aspect of the tectal plate. To the extent visualized,

the pituitary appears to be relatively diminutive with a sagittal diameter of about 0.3 cm and this could represent a partially empty sella, which can be an incidental finding or be associated with idiopathic intracranial hypertension.

- C2-C3: Minimal broad-based disc bulge encroaching upon the anterior thecal sac. The AP diameter of the central spinal canal is not significantly narrowed. No significant foraminal narrowing.
- C3-C4: Minimal broad-based disc bulge just touching the anterior surface of the spinal cord as the patient is positioned; the AP central spinal canal diameter is not significantly narrowed. Mild right and mild/moderate left neural foraminal narrowing.
- C4-C5: Minimal broad-based disc bulge touching the anterior surface of spinal cord as the patient is positioned; the AP central spinal canal diameter is not significantly narrowed. Minimal left foraminal narrowing suggested.
- C5-C6: Minimal/mild broad-based diffuse disc bulge touching the anterior surface of the spinal cord as the patient is positioned; the AP central spinal canal diameter is not significantly narrowed. Suggestion of minimal left foraminal narrowing.
- C6-C7: Minimal/mild diffuse disc bulge with a superimposed component of a midline and left paramedian mild broad disc herniation, minimally deforming the anterior left paramedian aspect of the spinal cord, 30/36. Suggestion minimal inferior focal left paramedian subligamentous extension of the disc herniation extending about 0.3 cm inferior to the level of the disc space. Suggestion of bilateral moderate (marked) neural foraminal narrowing.
- C7-T1: Minimal broad left paramedian disc herniation minimally deforming the anterior left paramedian aspect of the thecal sac. Suggestion of minimal right foraminal narrowing.

**# Low back pain and Left-sided sciatic pain s/p Lumbar spine surgery due to MRI findings of:**

-1 mm retrolisthesis. 4-5 mm broad-based disc protrusion at L4-L5, causes moderate AP canal stenosis and contact and impression on the bilateral L5-S1 nerve roots at this level and causes moderate bilateral foraminal stenosis. Mild ligamentous hypertrophy. Mild to moderate bilateral facet arthrosis.

-2 mm retrolisthesis. 4-5 mm broad-based disc protrusion at L5-S1, eccentric to the left, causes mild AP canal stenosis and contact with the right S1-S2 nerve root at this level and contact and impression on the left S1-S2 nerve root at this level, and causes moderate right foraminal stenosis and contact with the right L5-S1 exiting nerve root, and moderate to marked left foraminal stenosis and contact and impression on the left L5-S1 exiting nerve root. Posterior endplate osseous spurring in the left lateral recess and left foramen, contributes to stenosis. Moderate right facet arthrosis. Moderate to marked left facet arthrosis, contributes to left foraminal stenosis

**Probable Duration of Care:**

Based on the diagnosis, current complaints, diagnostic studies and response to treatment, Ms. Xxxx's conditions are expected to gradually deteriorate with a lifetime duration of care of 32.2 years.

### **Projected Long-Term Medical and Rehabilitation Costs:**

Ms. Xxxx has residuals from the slip and fall dated 06/11/2023. Ms. Xxxx is expected to live 32.2 years more. Ms. Xxxx has an ongoing need for prescription medications including nortriptyline, gabapentin, hydroxyzine, methocarbamol, ubrevly, memantine, topiramate, and prazosin. (listed in Medical Cost Table), as well as scheduled and unscheduled provider appointments (listed in Medical Cost Table), and intermittent need for therapy for flare-ups of the conditions.

Ms. Xxxx has several documented diagnostic study findings, and physical exam findings for ongoing issues mild traumatic brain injury with sequelae that will require continued medications, imaging, and therapies in addition to cervical spine pain that will require interventional pain injections (listed in Medical Cost Table). Pre- and post-op diagnostic studies, perioperative medical and facility costs, and post-op therapy would be required (listed in Medical Cost Table).

Ms. Xxxx has chronic pain and will benefit from prolonged use of a TENS unit that is expected to reduce pain levels, spasms, and improve functionality. The unit will be used regularly on an ongoing basis with a “useful life” of each unit being ten years before needing to be replaced (listed in Medical Cost Table).

Ms. Xxxx’s original injury was 27 months ago, and Ms. Xxxx has had occipital nerve block, lumbar spine surgery, and trigger point injections procedures over this time and has chronic persistent pain that is causing functional losses. The pain, in combination with significant functional losses, has resulted in the onset of reported symptoms of anxiety, depression, insomnia and fear/avoidance behaviors. These symptoms are a late consequence of the injury and need to be evaluated and treated. In addition, worry about financial barriers to treatment, and concerns about further injections and surgery need to be addressed. Therefore, according to current treatment Guidelines (ODG), evaluation and treatment from a Functional Restoration Program will be necessary. This program is provided after Ms. Xxxx completes all necessary treatment and is in addition to the supportive psychotherapy sessions and psyche medication management that will be required while Ms. Xxxx is undergoing treatment (listed in Medical Cost Table).

Ms. Xxxx has had objective need for long-term use of a lumbosacral orthotic and cervical decompression collar. It will be used regularly on an ongoing basis with a “useful life” being five years before needing to be replaced (listed in Medical Cost Table).

### **Potential Medical Costs Not Included in the Medical Cost Table:**

Medical costs not documented in the medical cost table could be incurred as a result of the condition gradually worsening over many years. Also not included are the medical costs associated with unexpected complications from surgery or procedures. These medical costs are not reasonably medically probable, but would not be considered unexpected for the conditions in question.

Ms. Xxxx is expected to have cervical and/or lumbar surgery which may ‘fail’ and result in failed spinal surgery syndrome that would require lifelong spinal cord stimulation and need for annual maintenance and battery replacements.

Ms. Xxxx may develop adjacent segment syndrome or recurrent disc herniation and/or arthritic changes, resulting in the need for repeat surgery.

Life Care Plan - [REDACTED]

Ms. Xxxx has had a significant head injury that may continue to progress with continually CNS degeneration and loss of executive and cognitive functioning such that she require 24/7 assisted living care in a memory facility.

# Medical Cost Table

|                                         |    |          | Unit<br>Cost | Lifespan<br>Years | Annual<br>Cost | Lifetime<br>Cost | Guideline                                    | sub |
|-----------------------------------------|----|----------|--------------|-------------------|----------------|------------------|----------------------------------------------|-----|
| <b>Drug Costs:</b>                      |    |          |              |                   |                |                  |                                              |     |
| Nortriptyline 50mg                      |    |          | \$22 /month  | 32.2              | \$264          | \$8,500          | <a href="http://goodrx.com/">goodrx.com/</a> |     |
| Gabapentin (Neurontin) 300mg            |    |          | \$13 /month  | 32.2              | \$156          | \$5,023          | <a href="http://goodrx.com/">goodrx.com/</a> |     |
| Hydroxyzine (Orgatrx) 25mg              |    |          | \$8 /month   | 32.2              | \$95           | \$3,072          | <a href="http://goodrx.com/">goodrx.com/</a> |     |
| Methacarbamol (Robaxin) 500mg           |    |          | \$35 /year   | 32.2              | \$35           | \$1,127          | <a href="http://goodrx.com/">goodrx.com/</a> |     |
| Ubrevely 100mg (8 tabs per month)       |    |          | \$895 /month | 32.2              | \$28,819       | \$927,972        | <a href="http://goodrx.com/">goodrx.com/</a> |     |
| Memantine 10mg bid                      |    |          | \$23 /month  | 32.2              | \$276          | \$8,887          | <a href="http://goodrx.com/">goodrx.com/</a> |     |
| Topiramate 25mg                         |    |          | \$8 /month   | 32.2              | \$96           | \$3,091          | <a href="http://goodrx.com/">goodrx.com/</a> |     |
| Prazosin 5 mg                           |    |          | \$20 /month  | 32.2              | \$240          | \$7,728          | <a href="http://goodrx.com/">goodrx.com/</a> |     |
| <b>Provider Visits:</b>                 |    |          |              |                   |                |                  |                                              |     |
| Orthopedist 99214                       | 4  | annual   | \$261        | 32.2              | \$1,044        | \$33,617         | FAIR                                         | ODG |
| Psychiatrist 99214                      | 4  | annual   | \$302        | 5                 | \$1,208        | \$6,040          | FAIR                                         | ODG |
| Neurologist 99214                       | 2  | annual   | \$302        | 32.2              | \$604          | \$19,449         | FAIR                                         | ODG |
| Psychotherapy 90837                     | 50 | lifetime | \$290        |                   |                | \$14,500         | FAIR                                         | ODG |
| Vestibular Therapy 97112                | 1  | lifetime | \$280        |                   |                | \$280            | FAIR                                         | ODG |
| TBI Rehab Program                       | 1  | lifetime | \$53,500     |                   |                | \$53,500         | McGregor et al                               | ODG |
| Physical Therapy                        | 12 | annual   | \$205        | 32.2              | \$2,460        | \$79,212         | HealthCost                                   | ODG |
| <b>Diagnostic Studies:</b>              |    |          |              |                   |                |                  |                                              |     |
| Comprehensive Metabolic Profile         | 2  | annual   | \$28         | 32.2              |                | \$1,803          | Quest                                        | ODG |
| Brain MRI without contrast 70551        | 5  | lifetime | \$2,053      |                   |                | \$10,265         | FAIR                                         | ODG |
| VAT/ENG testing for Vertigo             | 5  | lifetime | \$3,201      |                   |                | \$16,005         | FAIR                                         | ODG |
| EEG standard 95816                      | 5  | lifetime | \$997        |                   |                | \$4,985          | FAIR                                         | ODG |
| Cervical MRI w/o contrast 72141         | 5  | lifetime | \$2,395      |                   |                | \$11,975         | FAIR                                         | ODG |
| NCV Upper Extremity 95908 (each)        | 20 | lifetime | \$637        |                   |                | \$12,740         | FAIR                                         | ODG |
| EMG Upper Extremity 95869 (each)        | 20 | lifetime | \$417        |                   |                | \$8,340          | FAIR                                         | ODG |
| Lumbar MRI w/o contrast 72148           | 5  | lifetime | \$2,667      |                   |                | \$13,335         | FAIR                                         | ODG |
| NCV Lower Extremity 95866 (each)        | 20 | lifetime | \$615        |                   |                | \$12,300         | FAIR                                         | ODG |
| EMG Lower Extremity 95912 (each)        | 20 | lifetime | \$368        |                   |                | \$7,360          | FAIR                                         | ODG |
| Neuropsychological Comp (8 hrs)         | 5  | lifetime | \$3,340      |                   |                | \$16,700         | FAIR                                         | ODG |
| <b>Durable Medical Equipment (DME):</b> |    |          |              |                   |                |                  |                                              |     |
| LSO brace                               | 5  | lifetime | \$70         |                   |                | \$350            | Rinellapo                                    | ODG |
| Cervical decompression collar           | 5  | lifetime | \$20         |                   |                | \$100            | Amazon                                       | ODG |
| TENS unit                               | 3  | lifetime | \$450        |                   |                | \$1,350          | rsmed                                        | ODG |

|               |   |        |      |      |       |         |          |     |
|---------------|---|--------|------|------|-------|---------|----------|-----|
| TENS supplies | 2 | annual | \$60 | 32.2 | \$120 | \$3,864 | TENSpros | ODG |
|---------------|---|--------|------|------|-------|---------|----------|-----|

**Procedures**

**(surgeries)**

None

**(injections)**

|                                     |   |       |         |  |  |          |      |     |
|-------------------------------------|---|-------|---------|--|--|----------|------|-----|
| Cervical Epidural Steroid injection | 6 | 62321 | \$3,345 |  |  | \$20,070 | FAIR | ODG |
|-------------------------------------|---|-------|---------|--|--|----------|------|-----|

**Functional Medicine:**

|                                                                     |   |          |          |  |  |          |      |     |
|---------------------------------------------------------------------|---|----------|----------|--|--|----------|------|-----|
| Functional Restoration Program<br>(3 day evaluation plus 160 hours) | 1 | lifetime | \$30,000 |  |  | \$30,000 | FAIR | ODG |
|---------------------------------------------------------------------|---|----------|----------|--|--|----------|------|-----|

|                                   |           |
|-----------------------------------|-----------|
| <b>Total Cost of Drugs:</b>       | \$965,400 |
| <b>Total Cost of Providers:</b>   | \$206,598 |
| <b>Total Cost of DME</b>          | \$5,664   |
| <b>Total Cost of Diagnostics:</b> | \$115,808 |
| <b>Total Cost Procedures:</b>     | \$20,070  |
| <b>Total Cost Pain Psych:</b>     | \$30,000  |

|                                |             |
|--------------------------------|-------------|
| <b>Total Medical Expenses:</b> | \$1,343,540 |
|--------------------------------|-------------|

**Drug Costs -** Certain drugs will only be used for flare-ups. These annual costs are based on two flare-ups a year for an average of 21 days per flare-up.

**DME costs** are highly variable based on the brand, model, and place of purchase. The costs provided in the table are considered average or median prices for the type of DME.

**Orthopedist' under Providers** refers to a physician that treats physical, sports, or traumatic injuries on as a regular part of the practice.

### **Medical Cost Table (MCT) Assumptions:**

In order to interpret and understand the Medical Cost Table (MCT) the following assumptions have been made based on reasonable medical probability. Injuries to discs, or intra-articular joints, will continue to gradually degenerate over the years due to changes in alignment, structure, scarring and other factors.

Based on the nature of the injury and the response to the prior treatment, the injured party is expected to have two flare-ups per year on average. These flare-ups are expected to increase in intensity and frequency as the injured party ages and degenerative processes progress. There may be periods early on with few or no flare-ups, but on average over the lifetime of 32.2 years the injured party will have 2 flare-ups per year.

The injured party will have flare-ups that cannot be managed, and the injured party will seek medical assistance. Each flare-up will require an initial orthopedist visit and a follow-up visit about three weeks later. The orthopedist will treat the injured party with PT and medications over a three-week period. In the MCT, for each flare-up the injured party will have 2 orthopedist visits, 6 sessions of PT and 21 days of medication.

Most flare-ups will respond to the above treatment. However, some flare-ups will not respond to the three weeks of treatment and therapy and will require a diagnostic study or therapeutic block to quiet the flare-up down.

**Disclaimer: This plan is based on past, present, and anticipated medical needs. Projections are based on the review of the medical records available at the time of this report. Costs are based on present-day values.**



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### **Primary MCT References:**

ODG – Official Disability Guidelines: Evidence-Based Decision Support Medical Treatment Guidelines.

This is a professional access database found at [www.ODGbyMCG.com/treatment](http://www.ODGbyMCG.com/treatment). This website provides Medical Treatment Guidelines (MTGs) for the treatment of injuries. The Guidelines are determined using Evidence-Based Medicine (EBM) with is a methodology whereby peer-reviewed medical literature is reviewed regularly to determine the best practices for the use of diagnostic and treatment modalities. The ODG is specifically designed for the treatment of injuries. The ODG is used in the MCT to determine where a diagnostic study or treatment is ‘medically necessary’ to treat the condition now or in the future.



FAIR - [www.FairHealth.org](http://www.FairHealth.org) – Cost by CPT and zip code.

This is a professional access database found at [www.FairHealth.org](http://www.FairHealth.org). This website provides the actual retail or billed amounts from the clinical practice for each 5-digit CPT code by zip code throughout the US. The 50% or 80% percentile cost are used as the standard recognized amounts for CLCP methodologies. This is the cost the clinical practice will actually bill for the CPT code.

GoodRx – [www.GoodRx.com](http://www.GoodRx.com) – Pharmaceutical costs

This is a consumer database for the actual consumer cost of prescription medications. The median (middle) price for the generic version (where available) of the medication is used to determine the cost of the medication in the MCT.

**Other references:**

[www.StatPearls.com](http://www.StatPearls.com) – NCBI Bookshelf (nih.gov)

[www.Orthobullets.com](http://www.Orthobullets.com) – Peer-reviewed clinical practice guidelines.

ICSI – Institute for Clinical Systems Improvement

AAPM – American Academy of Pain Management

Milliman – Milliman - Utilization and Cost of Lumbar Spinal Stenosis in Commercially Insured Population

RinellaPro - [www.Rinellapro.com](http://www.Rinellapro.com) - Rinella Prosthetics and Orthopedics

DMEDir - [www.DME-direct.com](http://www.DME-direct.com)

CEmag - Chiropractic Economics Magazine (Southern region)

Healix Labs - <https://healixpathology.com/laboratory-services.html>

HealthCost – [www.Health.costhelper.com](http://www.Health.costhelper.com) - non-surgical treatment program

Healthline – [www.Healthline.com](http://www.Healthline.com)

New Choice - <https://www.newchoicehealth.com/spinal-cord-stimulator-cost>

Organicell – [www.Organicell.com](http://www.Organicell.com)

Rsmed - [www.Rsmedical.com](http://www.Rsmedical.com) - (TENS and NMS units comparison prices)

TENSpros - [www.TENSpros.com](http://www.TENSpros.com) (accessory supply bundle)

GENWORTH - [www.genworth.com](http://www.genworth.com)

Moss C et al. Weaning From Long-term Opioid Therapy. Clin Obstet Gynecol. 2019 Mar; 62(1):98-109.

**References Specific to Life Care Planning Background, Methodologies, and Scope of Practice:**

Gonzales, JG. A Physician's Guide to Life Care Planning – Tenets, Methods, and Best Practices for Physician Life Care Planners, 1<sup>st</sup> Ed. ([A Physician's Guide to Life Care Planning: Tenets, Methods, and Best ... - Google Books](#))

May, VR. [et.al.](#) Handbook of Medical Aspects of Disability and Rehabilitation for Life Care Planning. ([Handbook of Medical Aspects of Disability and Rehabilitation for Life](#))

Rutherford-Owen, T. et. al. – Life Care Planning and Case Management – Across the Lifespan, 5<sup>th</sup>. Ed. ([Life Care Planning and Case Management Across the Lifespan | Tanya Rut](#))

## **Appendix A:**

### **Daubert Admissibility of Evidence Expert Issues:**

According to 1993 Daubert v. Dow the following five categories need to be complied with in order for an expert to have evidence considered to be admissible.

*Can the theory or technique be tested?*

The theories and methodologies used by all CLCP professionals have been tested and the results published in several peer-reviewed LCP Journals since 1992.

*Has the science been peer-reviewed?*

The following peer-reviewed textbooks establish the strong scientific foundation of the LCP methodology used to formulate this LCP and espoused by the ICHCC and the American Academy of Physician Life Care Planners (AAPLCP).

Gonzales JG, A Physician's Guide to Life Care Planning – Tenets, Methods and Best Practices for Physician Life Care Planners. AAPLCP (2017)

May VR, Bowman R, Barna S, Handbook of Medical Aspects of Disability and Rehabilitation for Life Care Planning. (2024)

Rutherford-Owen, T, Barros-Bailey M, Weed RO, Life Care Planning and Case Management (Fifth Edition), ICHCC (2024).

*Is the rate of error acceptable?*

For all methodologies and statistical calculations standard 95% confidence limits are applied. This is the same rate of error used in all medical and laboratory sciences. Only medical services, studies or equipment that will be required with 'medical certainty' are included in the Medical Cost Table (MCT)

*Does the methodology applied enjoy widespread acceptance?*

Certification in Life Care Planning (CLCP) and membership in the American Academy of Physician Life Care Planning (AAPLCP) are the standard credentials for LCP experts in all 50 states and federally.

*Is the opinion relevant to the issue?*

In this LCP Dr. Glazer will predict with a reasonable degree of medical probability, those medical services and medical equipment that will be necessary to evaluate or treat the injury over the remainder of the injured party's life.